THE LOOKING GLASS BEAUTY SCHOOL

Gel Polish Nails CPD Course — Learner Manual

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Document review: Version 2.0 — October 2026 | Review by October 2027 or sooner if law, products or guidance change

Continuing Professional Development (CPD) activity — not a regulated qualification.

Document control and permitted use

This manual has been prepared for The Looking Glass Beauty School's Gel Polish Nails CPD Course. It is designed to support qualified beauty or nail professionals in updating and demonstrating safe gel polish knowledge and practical competence. It does not replace product-specific training, an insurer's requirements, local-authority conditions, a Safety Data Sheet (SDS), or medical advice.

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> ***SCOPE BOUNDARY — This course teaches gel polish application, French and simple colour variations, and soak-off removal on healthy natural fingernails. Structural gels (including gel polish in a bottle), hard gel and liquid-and-powder acrylic are compared so learners can recognise the systems and work within scope; this CPD course alone does not qualify a learner to provide those separate services. Extensions, structural overlays and electric-file work on the natural nail are not taught or assessed here.***

Course author

I am Jessica Jane Preston, a qualified nail professional, beauty educator and salon co-founder with more than fourteen years of industry experience. I hold a VTCT Level 2 NVQ Diploma in Nail

Services and an Award in Education and Training. Since 2021 I have delivered classroom-based beauty education and have trained more than 400 learners. My CV and qualification certificates are available from the school on request.

Important professional boundaries

- A nail professional observes, records, adapts, postpones and refers; a nail professional does not diagnose a disease or prescribe medical treatment.
- Never apply uncured product to skin as a 'patch test'. Suspected acrylate allergy requires medical assessment and, where appropriate, formal patch testing by a suitably qualified clinician.
- Do not rely on this manual to override a product label, SDS, manufacturer system guide, local licence condition or insurer instruction.
- Where an urgent reaction includes breathing difficulty, collapse, rapidly increasing facial or throat swelling, or other signs of a medical emergency, call 999. For non-emergency concerns use a pharmacist, GP or NHS 111 as appropriate.

Chapter 1 — Course specification and learning journey

What learners can expect, who the course is for, and how competence is assessed

Course title

Gel Polish Nails CPD Course

> ***A NOTE ON NAMES — Clients often ask for 'Shellac' when they mean any gel polish. Shellac is a registered trade mark of one manufacturer and describes that manufacturer's system only. This course is titled and advertised as a gel polish course. In teaching and in client records, name the actual base, colour and top coat used, and the lamp, rather than a brand the client happens to know.***

Course description

The Gel Polish Nails CPD Course is a practical skills-development activity for existing beauty and nail professionals. Learners revisit the anatomy of the nail unit and the conditions that require a service to be adapted, postponed or refused; explain how gel polish cures, why it is applied in thin coats, and what lamp compatibility and under-curing mean for the client's skin; and learn to prepare the natural nail, apply base, colour and top coat in thin, fully cured, capped coats, produce a French finish and simple colour variations, and remove gel polish safely by soaking. The course also covers allergy prevention, equipment care, UK legal responsibilities, aftercare and honest advertising.

Successful completion requires a theory assessment, a directly observed practical assessment and three satisfactory post-course case studies. This activity is Continuing Professional Development and does not replace or confer a regulated nail qualification.

> ***SCOPE BOUNDARY — This course teaches gel polish on healthy natural fingernails. Structural gels and liquid-and-powder acrylic are compared so learners can recognise the systems and work within scope; this course alone does not qualify a learner to provide those services. Extensions, structural overlays and electric-file work on the natural nail are not taught or assessed here.***

Entry requirements and prerequisites

- Learners must be aged 18 or over unless the school's insurer has confirmed an alternative minimum age in writing.
- Learners must hold at least a Level 2 Beauty Therapy, Nail Services or equivalent qualification that includes manicure and pedicure knowledge, nail structure and function, and safe salon practice. Equivalent prior learning may be accepted subject to provider and insurer approval.
- No previous gel polish experience is required. Confident manicure skills, including cuticle work and shaping, are assumed, because the course does not re-teach a manicure.
- Learners must be able to read and follow English-language product labels, Safety Data Sheet information, consultation documents and assessment instructions. Reasonable adjustments will be considered without removing competence or safety requirements.
- Learners with a known acrylate or methacrylate sensitivity must not undertake this course.
- Before offering the treatment commercially, learners must confirm that their qualification route, this CPD training and intended working arrangements are accepted by their own insurer and local authority.

Duration and delivery structure

Component	Time	Evidence
Pre-course reading and knowledge activity	1.5 hours	Completed learner workbook prompts
Tutor-led classroom and practical day	7.5 guided learning hours, excluding breaks	Attendance, questioning and direct observation
Three post-course case studies	Approximately 6 hours total	Consultation, treatment record, photographs with consent, reflection and educator feedback
Total learner effort	Approximately 15 hours	All evidence retained in the learner portfolio

The lesson plan for the taught day (document G05 in the Tutor and Assessor Pack) sets out the timetable and matches these hours: 7.5 guided learning hours on the day, 1.5 hours of pre-course reading and about 6 hours of case-study work, 15 hours in all.

Learning outcomes

- LO1 — Explain what gel polish is, its benefits, limitations and client suitability, and compare it with builder gel, hard gel and liquid-and-powder acrylic.
- LO2 — Identify the structures of the nail unit and the surrounding skin, and recognise conditions that require the service to be adapted, postponed or refused.
- LO3 — Conduct and record a professional consultation, obtain informed consent and create an individual treatment plan.
- LO4 — Explain photoinitiators, polymerisation, lamp compatibility, under-curing and the inhibition layer, and why gel polish is applied in thin coats.
- LO5 — Select, handle, store and dispose of products and equipment in accordance with risk assessment, COSHH, manufacturer instructions and current UK requirements.
- LO6 — Prepare the natural nail without avoidable damage to the plate or living tissue.
- LO7 — Apply base coat, colour and top coat in thin, even, fully cured coats with a capped free edge and no skin contact.
- LO8 — Produce a French finish and simple colour variations to a commercial standard.
- LO9 — Remove gel polish safely by soaking, without damage to the natural nail, and recognise when another product needs a different method or a referral.
- LO10 — Provide aftercare, manage contra-actions and adverse reactions, and set honest expectations about wear.

Assessment strategy and pass requirements

Theory: a written paper of 38 questions totalling 50 marks, with partial credit for multi-part questions. The pass mark is 80% (40/50). Any answer that instructs a learner to cure product on skin, deliberately under-cure, ignore a client reporting heat, peel or prise gel polish off, file the natural nail with an electric file without separate training, or conceal an active infection must be corrected before a pass is awarded.

Practical: direct observation on a suitable adult model, requiring at least 80% overall and a pass in every safety-critical criterion.

Case studies: three complete services after the taught day, each with consultation, before and after images where separate consent is held, product and lamp record, treatment notes, aftercare and reflection.

Models: learners arrange their own suitable adult model for the practical assessment and confirm this with the school at booking. The model must have healthy natural fingernails, no contraindication from Chapter 4, and must complete Form A. A learner who cannot bring a model must tell the school at least seven days before the course.

Where the assessment papers are: the theory paper (document G02), its mark scheme (G03) and the practical observation marking sheet (G04) are held in the Tutor and Assessor Pack and

are not printed in this manual. The practical is marked on twenty criteria; the seven below are safety-critical and every one must be met for a pass, whatever the overall score:

- No uncured product left on skin at any point; a hairline gap kept at the proximal and lateral folds.
- The compatible lamp and the full manufacturer cure time used for every coat; no shortened cures, no substitute lamp, no thick coat cured to save time.
- The client told before the first cure to report any heat at once, and the hand taken out of the lamp immediately if heat is reported.
- No cutting of living tissue; no filing into the lateral folds; the natural nail de-shined, not thinned.
- The service refused or adapted where a contraindication is present; suspected infection or reaction never covered.
- Removal by soaking as Chapter 14 describes; no peeling, prising, scraping or filing into the plate.
- Hygiene: single-use items not reused, tools decontaminated between clients, the blood or skin-break procedure followed if needed.

Chapter 2 — Gel polish: purpose, benefits and professional scope

Understanding the system before learning the technique

What gel polish is

Gel polish is a light-cured colour coating applied with a polish-style brush in thin coats: a base coat, two coats of colour and a top coat, each cured in a compatible lamp. It behaves like a traditional lacquer in the hand and like a gel under the lamp. The result is a hard, glossy finish that is dry the moment it leaves the lamp and wears for two to three weeks on a healthy natural nail.

What gel polish is not: it is not a structural product. It adds no length, no apex and no strength beyond a thin protective film. A client who wants strength or length needs a builder gel, hard gel or acrylic service and a professional trained in it. Keeping that boundary clear is part of being good at gel polish.

Benefits and limitations

Potential benefit	Professional limitation
Dry and scratch-proof the moment the hand leaves the lamp	Only if every coat was fully cured. A shortened cure gives a surface that looks dry over product that is not.
Two to three weeks of wear without chipping on a healthy nail	Wear depends on preparation, thin coats, a capped free edge and the client's hands. Never promise a number of days.
Thin, light and natural-looking	Thin is the technique, not a choice. A thick

	coat wrinkles, lifts and under-cures.
Removed by soaking, with no filing of the natural nail	Removal still takes 15 to 20 minutes and must be done properly; peeling it off takes nail plate with it.
Hundreds of colours, French, layering and simple art	Every product in the sequence must belong to one compatible system with its own lamp and cure times.
Suits almost every healthy natural nail	It does not suit a nail that is infected, separating, inflamed or sore, and it cannot hide one.

Who the treatment may suit

- A client with healthy natural fingernails who wants lasting colour and shine without the commitment of an enhancement.
- A client who finds ordinary lacquer chips within days, or who wants a finish that survives a holiday or an event.
- A client whose nails are short or bitten and who wants a tidy, protected finish while they grow.
- A client who understands that gel polish is removed professionally every two to three weeks and that picking it off damages the nail.
- A client whose consultation reveals no active contraindication.

When another service may be more appropriate

- Builder gel suits a client who wants strength, a true apex or a short extension on the natural nail.
- Liquid-and-powder acrylic or hard gel may suit a client who wants significant length or who is very hard on their hands.
- An ordinary manicure with lacquer suits a client who changes colour weekly or cannot return for professional removal.
- No coating of any kind should be used to conceal a suspected infection, unexplained nail change or active reaction.

Comparing the principal nail systems

System	How it hardens	Structure	Removal
Gel polish	Light cure in a compatible lamp	Thin colour coating; no structural apex, no added length	Soak off in acetone after breaking the top coat
Builder gel in a bottle	Light cure	Genuine apex; short extensions where the product allows	Product-specific: soak off, file off or both
Hard gel	Light cure	Rigid; overlays and extensions	File off; acetone does not normally remove it
Liquid-and-powder	Chemical reaction;	Strong; overlays and	Soak off after

acrylic	no lamp	full extensions	reducing bulk

NAME THE PRODUCT — A base coat, colour and top coat from one manufacturer, cured in that manufacturer's lamp for that manufacturer's times, is a tested system. Mixing brands is an untested experiment on a client's nails. Record the actual products and lamp used on every client card.

Where gel polish came from

Coloured nail lacquer became a mass-market product in the 1920s and 1930s, when car-paint chemistry was adapted for nails; it dried by evaporation and chipped within days. Light-cured gels for nails arrived in the 1980s as a lower-odour alternative to liquid-and-powder acrylic, but they were thick structural products, applied with a gel brush and removed by filing. Gel polish is the meeting of the two: a gel thin enough to apply like a lacquer and soak off like one, yet cured under a lamp so it leaves the salon hard and glossy. The first widely adopted systems reached salons in 2010, and the 'two-week manicure' changed what clients expected from colour. Within a few years every professional nail brand offered one, which is why the compatibility rule matters so much today: the products look alike and are not.

Service time, pricing and promotion

A full gel polish application on prepared natural nails commonly takes 45 to 60 minutes for a newly trained professional and 35 to 45 minutes with experience; a soak-off removal 15 to 20 minutes; a removal with re-application 60 to 75 minutes; a French finish adds 10 to 15 minutes. Price the service on your time, product cost and your market, not on what a competitor charges. The points below are a starting framework; set your own figures.

- Gel polish application: priced above a lacquer manicure, because of the product, the lamp and the wear the client gets.
- Removal: a separate charge, reduced or waived when the client rebooks a re-application.
- French finish, layered colour or simple art: a supplement on top of the application price.
- Repair of a single nail within the first few days: at your discretion, and only once the cause has been looked at.

Promotion that works: photograph every set from above and from the side, with consent and in the same light; a side view that shows a thin, even, capped coat is the picture other professionals notice. Describe the service honestly in line with Chapter 10 (lasting colour, never 'guaranteed chip-free'). Book the removal or re-application at the end of every appointment, two to three weeks ahead: it protects the natural nail and keeps the diary full. Tell clients plainly who gel polish is for and who it is not for; the client who leaves with the right service is the one who comes back.

Chapter 3 — Nail anatomy, physiology and growth

The structures a nail professional must protect

The nail unit is more than the visible plate. It includes living skin, growth tissue, protective folds and supporting structures. Understanding the difference between living and non-living tissue is essential because a cosmetic service should not deliberately cut, abrade or seal over damaged living tissue.

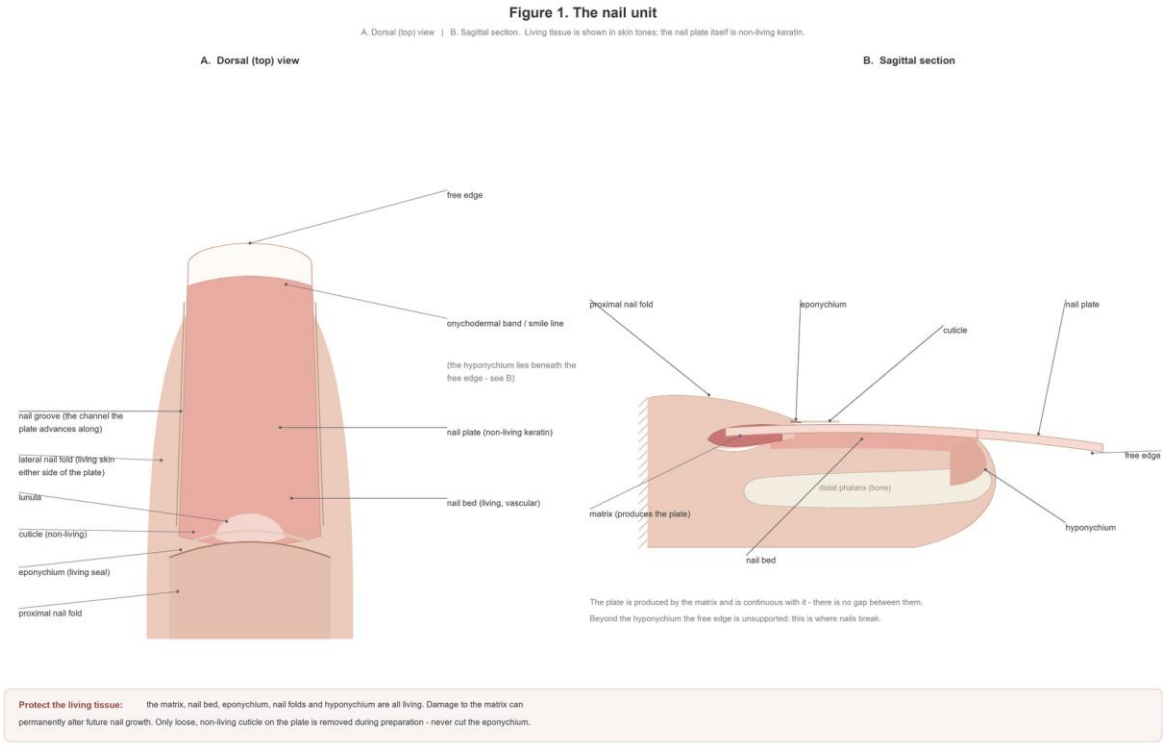


Figure 1. The nail unit. A: dorsal view. B: sagittal section. The matrix produces the nail plate and is continuous with it. The free edge beyond the hyponychium is unsupported.

Original illustration © The Looking Glass Beauty School, 2026.

Structures and functions

Structure	Function and professional relevance
Matrix	Living growth tissue beneath the proximal nail fold. Matrix cells divide, flatten and keratinise to form most of the nail plate. Significant injury can alter future nail growth; the matrix must never be deliberately filed, cut or probed.
Nail plate	The visible, hard but flexible keratin structure. It contains no blood vessels or nerves. Its layers can be thinned or delaminated by over-filing, peeling product,

	trauma and repeated dehydration.
Nail bed	Living, vascular tissue beneath the plate that supports and anchors it as it advances. It contributes to the pink appearance of a healthy attached nail. Separation of plate from bed is onycholysis.
Proximal nail fold / proximal nail wall	Living skin at the base of the nail that protects the emerging plate and matrix. It is not 'dead cuticle' and should not be aggressively pushed back or cut.
Lateral nail folds / nail walls	Living skin along each side of the plate. They protect the nail unit and guide the plate. Filing into these folds can cause pain, breaks in the barrier and infection risk.
Cuticle	Thin non-living tissue shed from the underside of the proximal nail fold and carried onto the plate. Only loose or adhered non-living tissue is gently removed during preparation.
Eponychium	Living tissue associated with the proximal nail fold and protective seal. It is often incorrectly called the cuticle. Do not cut it simply to create a visually 'clean' margin.
Lunula	The visible pale crescent of the distal matrix on some nails, most obvious on thumbs. Its visibility varies and absence from view is not automatically abnormal.
Hyponychium	Living tissue beneath the free edge where the nail bed ends. It forms a protective barrier. Do not dig under, cut or separate it while cleaning or shortening the nail.
Free edge	The part of the plate extending beyond the fingertip. It can be shortened and shaped while the finger and sidewalls are supported.
Nail grooves	Channels at the sides in which the plate advances. Product should not be flooded into these areas.
Onychodermal band	The transition between nail bed and hyponychium, sometimes visible as a narrow colour change beneath the plate. It is part of the distal protective seal.

How the nail grows

New nail-plate cells are produced in the matrix. As cells mature they flatten, lose their nuclei, fill with keratin and are pushed distally over the nail bed. A fingernail commonly grows at around 3 mm per month, although the rate varies between people, fingers and seasons. A completely

new fingernail may take roughly four to six months to grow from matrix to free edge. Growth may be slower with age, illness, impaired circulation, nutritional deficiency, some medication and significant trauma. Use a range and explain variation rather than promising an exact regrowth date.

Factors that can influence nail appearance and growth

- **Local trauma:** crushing, repeated tapping, biting, picking, over-filing and forced product removal can affect the plate, bed or matrix.
- **Water and chemicals:** repeated wetting/drying, detergents and solvents can contribute to brittleness and delamination.
- **General health:** illness, fever, inflammatory skin disease, anaemia and other conditions may be associated with nail changes, but a nail professional must not diagnose the cause.
- **Medication and medical treatment:** some medicines and treatments can change growth, colour, thickness or fragility. Record only information relevant to treatment safety and refer when the change is new or unexplained.
- **Age and individual variation:** ridging, thickness and growth rate vary. A difference is not automatically disease, but a sudden or marked change should not be concealed.
- **Product and technique:** incorrect preparation, incompatible lamps, skin contact, thick coats, aggressive removal and excessive enhancement length can damage or destabilise the service.

Protecting the natural nail

- Use the least abrasive preparation permitted by the product manufacturer. Do not 'etch' grooves into the plate.
- Support the finger and use light, controlled strokes. Stop if the client reports heat, pain or tenderness.
- Keep uncured gel away from all skin and from the underside of the free edge unless the system specifically directs otherwise.
- Never use the coating as a tool to prise, scrape or pick. Preventing mechanical trauma is part of aftercare.
- If the nail plate is visibly thin, sore or over-filed, postpone coating and allow recovery rather than hiding the damage.

Chapter 4 — Nail diseases, disorders and recognition

Observe, record, decide and refer — never diagnose

A disease is a pathological process such as infection or inflammatory illness. A disorder or nail change may be traumatic, congenital, behavioural or associated with general health. The categories overlap, and appearance alone is not enough to diagnose. A nail professional should

describe what is seen, ask relevant questions, decide whether treatment is safe and refer when uncertain.

> ***FOUR-STEP DECISION — OBSERVE the nail and surrounding skin. ASK when it began, whether it hurts or itches, and whether it is changing. DECIDE whether to proceed, adapt or postpone within your competence and insurance. RECORD the decision and REFER when there is infection, inflammation, unexplained change, pain, broken skin or uncertainty.***

Core diseases and disorders

Condition or change	What may be observed	Professional action
Onychomycosis (fungal nail infection)	Thickened, brittle, crumbly or discoloured nail; debris; lifting may occur. Psoriasis and trauma can look similar, so visual inspection is not a diagnosis.	Do not apply or reapply gel polish. Avoid cross-contamination, use single-use items appropriately and advise the client to seek pharmacist, GP or podiatry advice. Treatment is medical, not a salon service.
Onycholysis	Separation of the nail plate from the bed, often beginning distally. The detached area appears white, grey or discoloured. Causes include trauma, psoriasis, fungal infection, irritants and other conditions.	Do not cover active, extensive, painful, discoloured or unexplained separation. Keep the area dry, avoid probing under the plate and refer. A stable, diagnosed residual change may be treated only with insurer approval and no contraindication.
Onychophagy (nail biting)	Short, irregular free edge; surrounding skin may be picked, sore or broken. Severity varies.	Gel polish may be applied where the skin is intact and the plate is healthy; refuse where the surrounding skin is broken, sore or infected. A short, tidy gel polish service often helps a client who is trying to stop biting. Shape conservatively and cap the free edge well.
Nail psoriasis	Pitting, thickening, colour change, crumbling, onycholysis or oil-drop/salmon patches may occur. It is inflammatory and not contagious.	Do not diagnose. Avoid treatment during an active painful flare, where skin is broken, or where the presentation is uncertain. Stable diagnosed psoriasis may sometimes be treated

		with medical/insurer guidance and gentle technique.
Paronychia	Red, swollen, warm and tender nail fold; pus may be present. It may be acute bacterial infection or chronic inflammation associated with repeated wet work.	Refuse treatment. Do not drain, cut, massage or cover the area. Advise prompt pharmacy/medical assessment, especially if pain, pus, spreading redness or systemic illness is present.
True pterygium	Scar-like adhesion or abnormal forward growth of tissue involving the nail unit. Dorsal pterygium can split or narrow the plate; pterygium inversum affects tissue beneath the free edge.	Do not confuse true pterygium with ordinary cuticle. Do not cut or push adherent living tissue. Refer for medical assessment before cosmetic coating.
Onychogryphosis (thick dystrophic nail)	Marked thickening, curvature and distortion of the nail, which may trap debris or be difficult to cut. It usually affects toenails and is often associated with pressure, age or neglect.	Gel polish is inappropriate. Do not aggressively file or coat. Refer to a podiatrist/medical professional, especially where diabetes, poor circulation, pain or infection risk is present; do not attempt reduction unless separately qualified and insured.
Brittle or splitting nails (onychoschizia/onychorrhexis)	Peeling layers, transverse splitting or longitudinal ridges/splits. Repeated water/chemical exposure and trauma can contribute; systemic causes are also possible.	Mild stable brittleness with intact skin can usually be coated: a flexible base coat, thin coats, a capped free edge and gentle soak-off removal protect the plate. New, severe, painful or multiple-nail changes require referral. Do not promise that gel polish will cure the cause.
Koilonychia (spoon nails)	A concave or spoon-shaped plate. It can be congenital, traumatic or associated with health conditions such as iron deficiency.	If new or unexplained, refer. Do not use product to disguise a possible health sign. If longstanding, medically reviewed and intact, treatment may be adapted within insurer conditions.
Duckbill / flared nail shape	The free edge widens or fans	No structural issue for gel

	outward. This may be the natural growth pattern or result from previous enhancement architecture.	polish, which adds no length. Shape the free edge conservatively, cap it well and keep the length practical.
Leukonychia (white spots)	Small opaque white marks commonly follow minor matrix trauma and grow out with the plate. Widespread, powdery or changing white areas can have other causes.	A few stable trauma-related spots on an otherwise healthy nail can usually be coated. Do not diagnose. Refer powdery, spreading, lifting, painful or unexplained changes.
Beau's lines	Transverse depressions caused by temporary interruption of nail growth. They may follow illness, injury or other stress.	If shallow, growing out and the nail is healthy, a gentle cosmetic service may be possible. New, deep, multiple or unexplained lines should be referred.
Splinter haemorrhage	Fine red-brown or black lines beneath the plate, often from minor trauma but sometimes associated with health conditions.	If clearly linked to minor trauma and growing out, record and monitor. Multiple, recurrent or unexplained marks require medical advice. Never cover a suspicious dark streak without referral.
Subungual melanoma warning sign	A new or changing dark longitudinal band, pigment spreading onto surrounding skin, nail destruction, bleeding or a persistent unexplained lesion.	Do not cover. Urgently advise GP assessment. Nail melanoma is uncommon, but delay caused by cosmetic concealment is unacceptable.

Recognising what you are looking at

The photographs below show presentations you may meet at the consultation. Use them to recognise and describe what you see — never to diagnose. Where a nail does not look like any of these, that is itself a reason to pause and refer.



Figure 2. Onychomycosis affecting two fingernails. Yellow-brown discolouration, thickening, crumbling of the plate and separation at the free edge. Do not apply or reapply gel polish — treatment is medical, not a salon service.

Photograph © Grook Da Oger, via Wikimedia Commons, CC BY-SA 3.0.



Figure 3. Onychomycosis more commonly affects toenails, as here: marked thickening, yellow-white crumbling and debris beneath the plate. Psoriasis and repeated trauma can look similar — visual inspection is never a diagnosis.

Photograph © James Heilman, MD, via Wikimedia Commons, CC BY-SA 3.0.



Figure 4. Onycholysis of the ring and little fingernails. The plate has separated from the bed beginning at the free edge, and the detached area appears pale and discoloured. Do not cover active, extensive or unexplained separation.

Photograph © CopperKettle, via Wikimedia Commons, CC BY-SA 3.0.

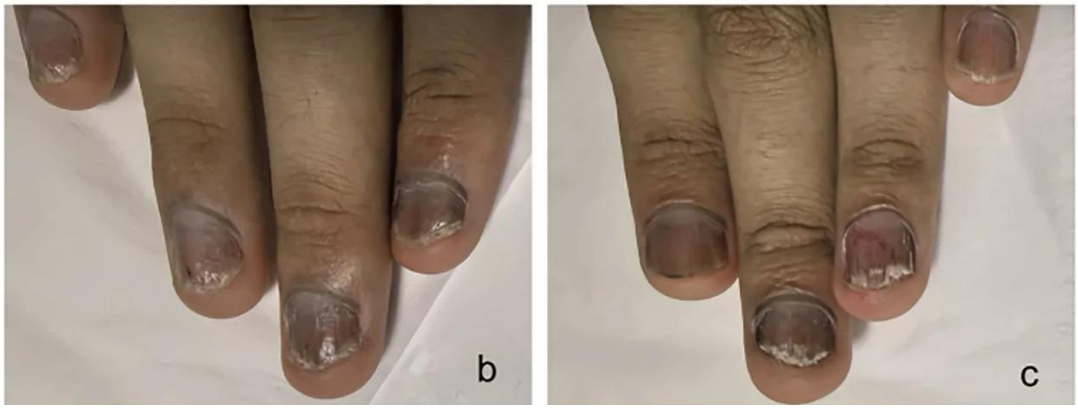


Figure 5. Nail psoriasis. Crumbling of the plate, subungual debris, colour change and separation at the free edge. Psoriasis is inflammatory and not contagious, but it can look very like fungal infection — which is exactly why appearance alone is never a diagnosis. Avoid treatment during an active flare, where skin is broken, or where the presentation is uncertain.

Photograph © Huang Y & Wen S, Frontiers in Immunology (2026), doi:10.3389/fimmu.2026.1941127, CC BY 4.0, cropped. Paediatric patient.



Figure 6. Acute paronychia. The nail fold is swollen, red and tender, with pus present. Refuse treatment. Do not drain, cut, massage or cover the area; advise prompt pharmacy or medical assessment.

Photograph © Mohammad2018, via Wikimedia Commons, CC BY-SA 4.0.



Figure 7. Chronic paronychia. The cuticle seal is lost, the proximal nail fold is thickened and bolstered, and the plate shows ridging and colour change. Commonly associated with repeated wet work.

Photograph © Daifallah M. Al Aboud, M.D., via Wikimedia Commons, CC BY 4.0.

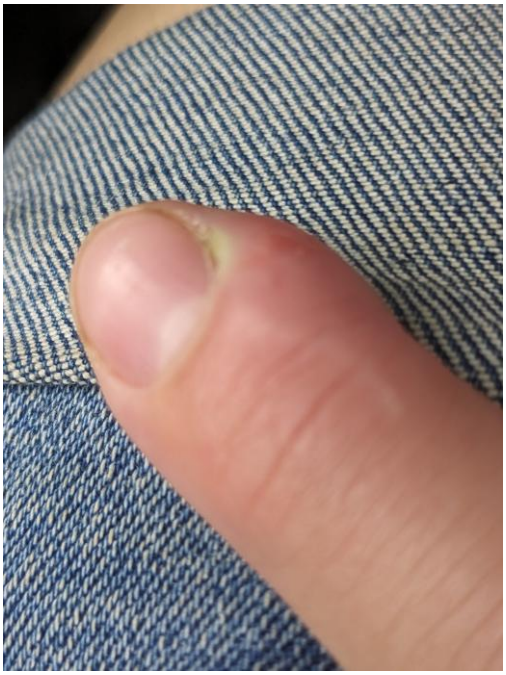


Figure 8. Early paronychia. Localised swelling and a small pus point at the nail fold, with the skin barrier already broken. Even at this stage the service is refused and the client referred.

Photograph © Hannah Clover, via Wikimedia Commons, CC BY 4.0.



Figure 9. Leukonychia punctata. A single opaque white spot following minor matrix trauma, which grows out with the plate. A few stable spots on an otherwise healthy nail can usually be coated — but do not diagnose.

Photograph © BrotherLongLegs, via Wikimedia Commons, CC BY-SA 4.0.



Figure 10. Longitudinal melanonychia — a dark band running the length of the plate. DO NOT COVER. A new or changing band, pigment spreading onto the surrounding skin, nail destruction or bleeding requires urgent GP assessment.

Photograph © Seungjun Lee & Seokchan Eun, via Wikimedia Commons, CC BY 4.0.

Green nail and suspected bacterial infection

Green, green-black or yellow-green discolouration beneath a lifted coating is commonly associated with colonisation by Pseudomonas bacteria in a moist space, although colour alone cannot confirm the organism. It is not correctly described as 'green fungus'. The professional response is not to hide the stain or prescribe a home chemical remedy.

1. Stop the service and do not apply fresh product over the discolouration.
2. If an existing enhancement can be removed without cutting skin or over-filing, remove it using the correct product method and within competence. If safe removal is uncertain, refer to a suitably qualified professional.
3. Keep the nail clean and dry. Do not dig under separated plate, buff away colour or use household bleach, neat vinegar, antibiotics or another treatment unless directed by a healthcare professional.
4. Advise the client to seek medical or pharmacy advice, particularly where there is pain, swelling, pus, spreading redness, diabetes, reduced immunity or persistent discolouration.
5. Discard contaminated single-use items, quarantine reusable tools and complete the full cleaning/disinfection or validated sterilisation process required by the risk assessment. Record the product, affected nails, observations, action and referral.

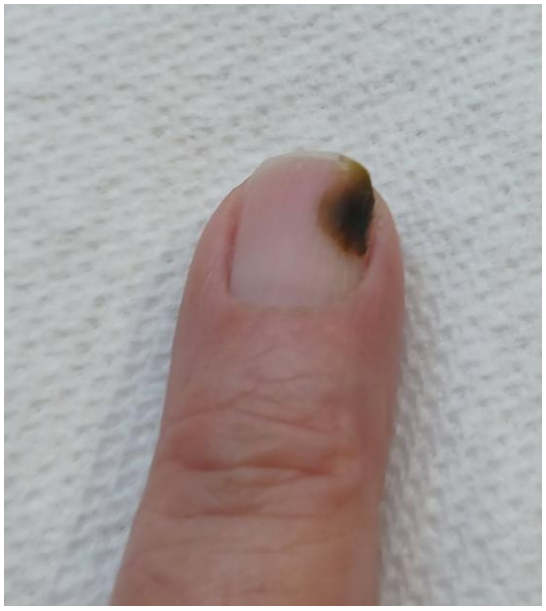


Figure 11. Green nail (chloronychia). Green-black discolouration associated with bacterial colonisation in a moist space. It is not 'green fungus'. Do not apply fresh product over the discolouration.

Photograph by Ekem, via Wikimedia Commons, CC0 1.0 (public domain dedication).



Figure 12. Green nail with associated onycholysis. The discolouration sits beneath a separated area of plate. Do not buff away the colour or use household remedies; keep dry, refer, and decontaminate tools.

Photograph by Ekem, via Wikimedia Commons, CC0 1.0 (public domain dedication).

Contraindications that are not diagnoses

The following findings or circumstances can prevent or restrict a gel polish service even when no named nail disease has been diagnosed:

- Known or suspected acrylate/methacrylate allergy, or a previous reaction to gel, acrylic, nail glue or adhesive products.
- Active eczema, dermatitis, psoriasis flare or unexplained rash on the hands, fingers or treatment area.
- Cuts, abrasions, bleeding, burns, blisters, swelling, heat, tenderness or recent trauma around the nail.
- A wart or other contagious lesion where the service could touch or spread it.
- Recent surgery, severe nail damage, significant plate thinning or pain on light pressure.
- A new, changing, unexplained or suspicious colour/shape change.
- A client unable to keep hands safely in the lamp, follow aftercare, communicate discomfort or give informed consent.
- Any medical condition, treatment or medication for which the insurer or manufacturer requires medical advice, adaptation or refusal. Diabetes and pregnancy are not automatic universal bans; assess individually, avoid injury and follow insurer/product guidance.

> ***DO NOT USE GP LETTERS AS A BLANKET TRANSFER OF RESPONSIBILITY — A doctor's note does not make an unsuitable treatment safe and does not override the professional's competence, product instructions or insurance. Ask only for relevant confirmation, make your own treatment decision and record the rationale.***

Chapter 5 — Consultation, treatment planning and consent

A safe service begins before the product is opened

Purpose of the consultation

A consultation is a structured two-way conversation and assessment. It establishes what the client wants, whether the treatment is appropriate, how it should be adapted and whether the client understands costs, maintenance, limitations and risks. A signed form supports the process but is not a substitute for observation, professional judgement or dialogue.

The consultation sequence

1. Create privacy and explain why relevant questions are being asked. Confirm the client's identity, age and preferred communication needs.
2. Clarify the desired result: colour, finish, shape, length, event, workplace requirements, budget and expected maintenance.
3. Ask about previous nail services, wear, lifting, removal methods, reactions, allergies and whether any nails are painful or changing.
4. Collect only health and medication information that is relevant to treatment safety. Explain that health data is sensitive and how it will be protected.

5. Visually and manually assess the nail plate, free edge, folds, surrounding skin and each hand. Compare nails; do not conceal suspicious findings.
6. Identify contraindications and decide to proceed, adapt, postpone or refuse. Explain the reason respectfully without diagnosing.
7. Agree a realistic treatment plan, product system, compatible lamp, colour, shape, service time, price, removal/maintenance and aftercare.
8. Explain foreseeable risks including temporary dryness, heat sensation, lifting, chipping, allergy/sensitisation risk and harm from picking or forced removal.
9. Obtain treatment consent. Obtain separate, optional consent for identifiable photographs or marketing; refusal of marketing consent must not prevent treatment.
10. Record products, batch numbers where practical, lamp model, cure times, observations, adaptations and aftercare. Update the record at every appointment.

Treatment planning

A treatment plan converts consultation findings into a safe, individual service. It should be specific enough that another competent professional can understand what was agreed and what happened.

Planning area	What to decide and record
Client goal	Colour/finish, occasion, desired wear, preferred shape and acceptable length
Nail condition	Plate thickness, flexibility, free-edge condition, skin integrity, relevant variations and contraindication decision
Lifestyle	Work, handwashing/wet work, chemicals, sport, gardening, caring, musical instruments, habitual picking and time available for maintenance
System	Base coat, colour and top coat from one compatible system, the compatible lamp and its cure schedule for each coat, and the removal method the system specifies
Adaptation	Shorter length, flexible base, reduced abrasion, one nail excluded, additional removal time, no cuticle nipping or treatment postponed
Service variation	Plain colour, French finish, layered or sheer colour, simple nail art within Chapter 13, removal only, or removal and re-application
Risk and consent	Benefits, limitations, allergy information, aftercare, cost and separate photo consent
Review	Expected maintenance/removal date, what the client should report, and outcome at the next visit

Consent is informed and ongoing

- The client must understand what will be done, material risks, alternatives and the right to stop. A signature does not remove the duty of care.
- Check consent again if the plan changes, discomfort occurs or another product is proposed.
- Do not use coercive wording such as 'I accept all responsibility'. A professional remains responsible for acting competently.
- For case-study photographs, obtain model consent for the educational purpose and specify who will see the images, how long they will be kept and whether the face/identity will be excluded.
- If consent cannot be freely and clearly given, do not treat.

Consultation scenario

> ***LEARNER ACTIVITY — A client books a gel polish manicure for a wedding on Saturday, says her last set 'peeled off in three days so I just pulled the rest off', and has a sore, reddened fold beside the index nail. Identify the immediate contraindication, what the peeling history tells you about her nails and the previous service, the questions you would ask, and a realistic plan for Saturday. Record your answer before discussing it with the group.***

Chapter 6 — Gel chemistry, curing science and heat

Why thin coats, product control and lamp compatibility protect clients

Monomers, oligomers and polymers

A monomer is a relatively small reactive molecule that can join with other molecules during polymerisation. An oligomer is a short pre-built chain containing reactive groups. A polymer is a much larger chain or network created when many reactive units join. Light-cured nail gels are normally formulated with a mixture of oligomers, monomers, photoinitiators, pigments and additives. The exact chemistry varies by product.

In a liquid-and-powder acrylic system, the powder is largely pre-polymerised particles and the liquid contains reactive monomer. In a gel system, the reactive mixture is supplied as a premixed gel and requires a suitable light source. This is why the terms 'monomer' and 'polymer' apply to both systems even though the application methods are different.

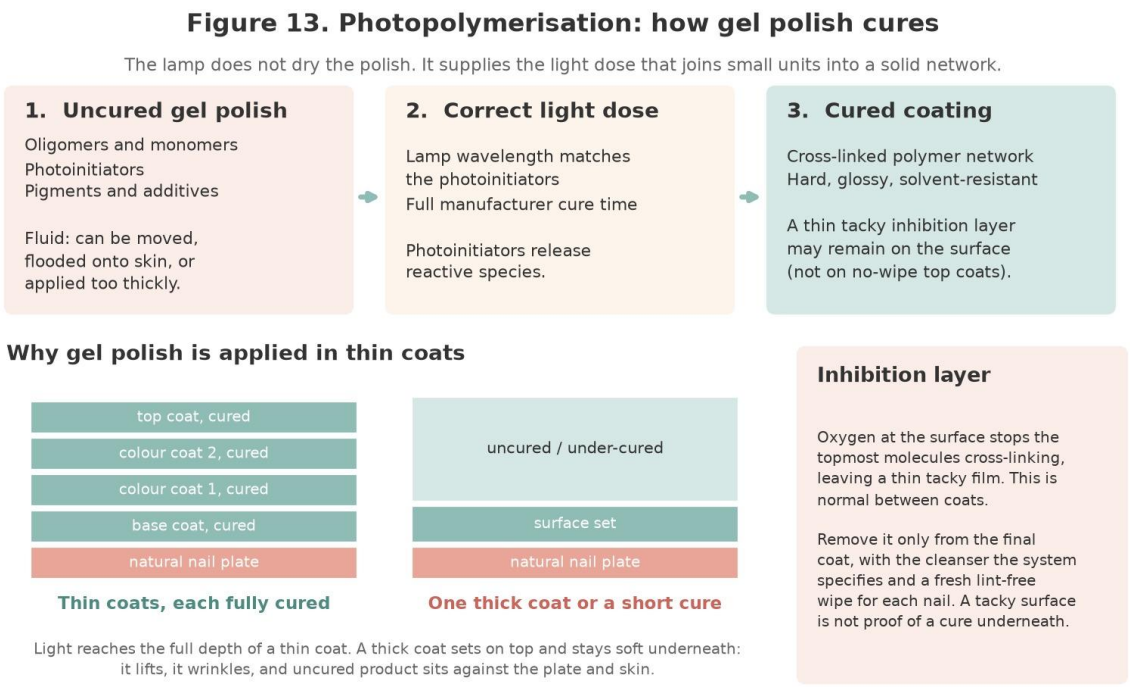


Figure 13. Photopolymerisation. The lamp supplies a light dose, not heat drying. Thin coats cure through their full depth; a thick coat or a shortened cure leaves uncured product against the plate and skin, which is the route to sensitisation.

Original illustration © The Looking Glass Beauty School, 2026.

What happens during curing

1. The compatible lamp emits light at wavelengths that the product's photoinitiators can absorb.
2. Activated photoinitiators generate reactive species that start chain growth and cross-linking.
3. The gel changes from a movable liquid/viscous material into a solid polymer network. This reaction releases heat.
4. A visually hard surface is not proof of adequate cure throughout the layer. Excess thickness, unsuitable wavelength, weak/dirty equipment, poor hand placement or shortened time can leave more residual reactive material.
5. Cure is a system property: product formulation, layer thickness, pigmentation, lamp output, exposure time and hand position all matter.

The inhibition layer

Many light-cured gels have a tacky surface after curing because oxygen at the surface inhibits part of the free-radical reaction. This is called the oxygen-inhibition layer. It is not evidence that all product beneath it is uncured and it is not accurately described as monomer being 'pushed to the top'. Depending on the system, the tacky surface either supports bonding of the next layer or is removed at the end using the specified cleanser. No-wipe top coats are formulated to finish without a noticeable tacky layer.

UV and LED lamps

Both traditional fluorescent UV lamps and modern LED nail lamps emit ultraviolet radiation used for curing. 'LED' describes the light source, not a completely different non-UV process. A lamp may emit around one or more wavelength bands, and the gel must be formulated for that output.

Requirement	Safe practice
System compatibility	Use the lamp identified or approved by the gel manufacturer. Do not assume a lamp will cure another brand because the bottle feels hard.
Wattage	The course uses a professional lamp of 48 W or higher where the selected system specifies it. Wattage is electrical power, not proof of wavelength or curing dose, so never select a lamp on watts alone; compatibility, wavelength, cure schedule and lamp maintenance always take priority.
Cure timing	Use the exact time for each product and layer. Do not shorten time to speed up the service and do not improvise extra-long cures as a substitute for thin coats and compatibility.
Hand position	Place fingers flat, centred and fully inside; reposition thumbs if the design requires. Crooked fingers can move nails away from the effective light area.
Maintenance	Keep reflective surfaces clean, inspect cable/plug/housing, follow bulb or device-life guidance, record faults and remove defective equipment from use.
Client information	Explain that lamps emit UV. Evidence on long-term skin-cancer risk from typical use is limited and evolving; discuss reasonable optional precautions without claiming zero risk.
Optional UV precaution	A client concerned about UV exposure can discuss fingerless UV-protective gloves or broad-spectrum sunscreen applied in advance, provided products do not contaminate the nail plate and the manufacturer's process is not compromised.

NEVER MIX AND HOPE — A base coat, colour, top coat and lamp are a chemical system. If a manufacturer has not confirmed cross-brand

compatibility, do not mix products or substitute the lamp during an assessed treatment.

Under-curing and sensitisation risk

Uncured and inadequately cured (meth)acrylate ingredients are more likely than a fully cured polymer to contact and sensitise skin. Repeated exposure can lead to allergic contact dermatitis. Once sensitised, a person may react to very small amounts and cross-reactions can affect future dental or medical materials. Prevention centres on keeping uncured product off skin, using compatible systems, curing correctly and reducing occupational exposure.

- Apply thinly and evenly; remove every trace from skin before curing.
- Never use a gel product to perform a skin patch test. 'HEMA-free' does not mean acrylate-free or allergy-proof.
- Do not touch inhibition layers with bare fingers. Use lint-free wipes once and discard safely.
- Do not use hands or fingernails as tools for cleaning gel from brushes, bottles or equipment.
- Wear task-appropriate PPE identified by the COSHH assessment/SDS and change contaminated gloves immediately.
- Monitor technicians' skin and respiratory health; act early on itching, cracking, rash, eyelid/facial eczema, cough or wheeze.

The principal allergens: HEMA and di-HEMA TMHDC

HEMA (2-hydroxyethyl methacrylate) and di-HEMA TMHDC (di-HEMA trimethylhexyl dicarbamate) are the methacrylates most often implicated in gel nail allergy. Under Regulation (EU) 2020/1682, as retained in the GB Cosmetics Regulation, nail products containing these ingredients are restricted to professional use only and must be labelled "For professional use only" and "Can cause an allergic reaction". The two main sensitisation routes are uncured product touching skin and product that has been under-cured. The British Association of Dermatologists has warned of a UK (meth)acrylate allergy epidemic, which is why skin-contact avoidance and correct curing are treated as safety-critical throughout this course. Remember that 'HEMA-free' does not mean acrylate-free or allergy-proof: other reactive ingredients can also sensitise.

Photoinitiator regulation: TPO

TPO (trimethylbenzoyl diphenylphosphine oxide) is a photoinitiator that was formerly common in gel systems. It has been classified as a category 1B CMR substance (toxic to reproduction). It is banned in the EU and Northern Ireland from 1 September 2025, including continued professional use of existing stock. In Great Britain, the Cosmetic Products Regulation (EC) No 1223/2009 (Restriction of Chemical Substances) (Amendment and Transitional Provisions) Regulations 2026 (SI 2026/23) add TPO to Annex II (prohibited substances) with transition periods: no new TPO products may be placed on the GB market after 15 August 2026, and

existing compliant stock may continue to be made available and used until 14 February 2027. Because the law is in transition, professionals must check ingredient lists for TPO, verify reformulated products with their suppliers, and confirm the current legal position at each course review and before purchasing stock.

Heat during the cure

Polymerisation releases heat. In gel polish the coats are so thin that a client rarely feels it, which is exactly why a client who does feel heat must be taken seriously: it usually means a coat was too thick, the plate is thin or over-filed, or the lamp is more powerful than the product was written for. Builder gel, applied in thick structural layers, carries a far greater heat-spike risk; the response is the same.

1. Tell every client before you start that a nail can feel warm in the lamp and to say so at once; agree a stop signal so they do not have to speak.
2. If the client reports heat, have them take the hand out immediately. Do not ask them to endure it.
3. Let the nail cool completely, then return it to the lamp to finish the cure.
4. For the next nail apply a thinner coat and, if the lamp offers one, use a low-heat setting.
5. Record the event, the nail, the product, the lamp and what you changed.

> ***NEVER TELL A CLIENT TO SIT THROUGH IT — Heat under the nail is a warning. Taking the hand out and re-curing costs seconds; a burned nail bed can lift the plate from the bed and take months to grow out.***

Chapter 7 — The anatomy of a good gel polish application

What the finished nail must look like before you can produce it

A gel polish service succeeds or fails on four things that are invisible to the client and obvious to a professional: how thin the coats are, whether the free edge is capped, whether the margins are clean, and whether every coat was fully cured. None of them is about colour. A perfect colour on a thick, uncapped, flooded coat will peel in days.

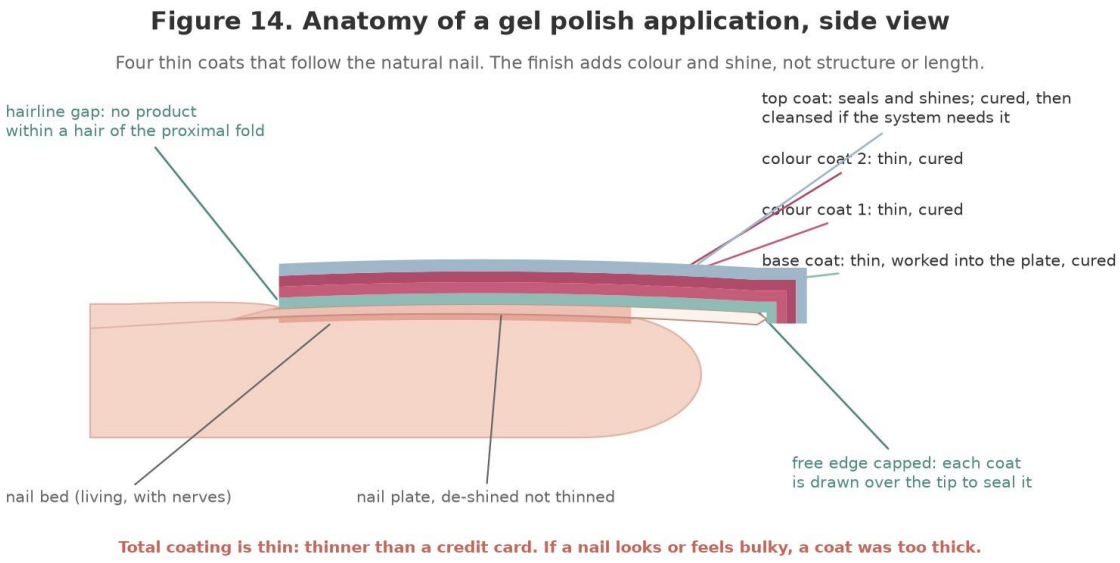


Figure 14. Anatomy of a gel polish application, side view. Four thin coats that follow the natural nail: base worked into the plate, two coats of colour, top coat. A hairline gap at the proximal fold, and every coat drawn over the free edge to cap it.

Original illustration © The Looking Glass Beauty School, 2026.

The four things a good application needs

Feature	What good looks like	What goes wrong without it
Thin coats	Each coat thin enough to be streaky on its own; the colour builds over two coats, not one	A thick coat sets on the surface and stays soft underneath: it wrinkles, lifts in a sheet and leaves uncured product against the plate and skin
Capped free edge	Every coat drawn across the tip of the nail so the edge is sealed	Wear starts at the free edge within days; the colour chips back from the tip
Clean margins	Colour to the sidewalls and a hair short of the proximal fold; nothing on skin	Product on skin lifts, looks untidy and is a direct sensitisation risk; a wide gap looks grown out on day one
Full cure, every coat	The compatible lamp, the manufacturer's time, the hand flat and still	Under-cure is invisible on the day and shows as peeling, dull colour and, over time, allergy

Thickness

The whole application, base to top coat, should be thinner than a credit card. Thin is not a stylistic choice: light must reach the bottom of every coat for it to cure, and the inhibition layer of a thin coat is the key the next coat bonds to. If a nail looks or feels bulky, a coat was too thick; take it off and start that nail again rather than hoping the top coat will hide it.

Checking before the cure

A fault in an uncured coat takes two seconds to fix with the brush. The same fault once cured can only be filed off. Make the three checks below on every nail, every coat, before the hand goes in the lamp.

Figure 15. Check every nail before it goes in the lamp

Three looks, in this order, on every coat. Once it is cured, a fault can only be filed off.

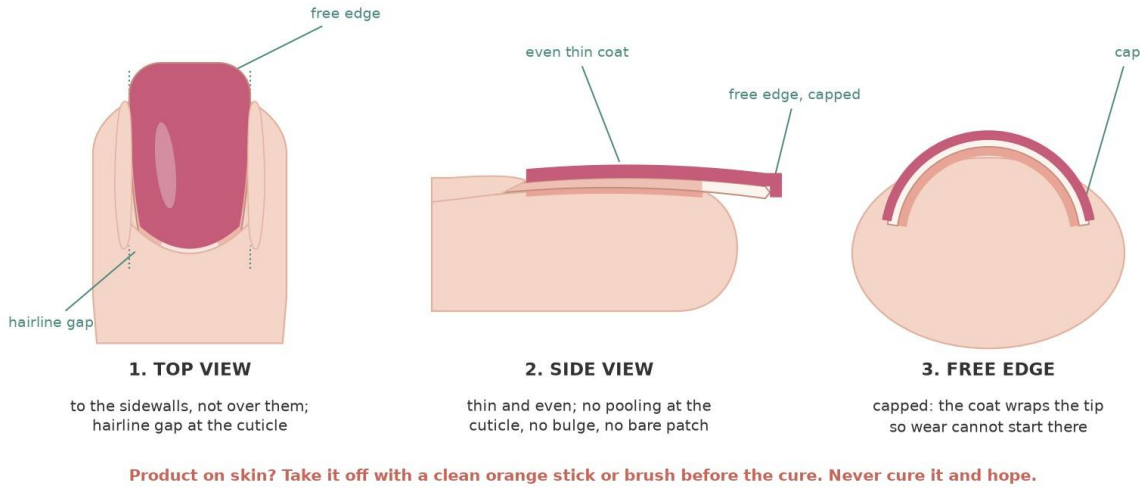


Figure 15. Check every nail before it goes in the lamp: top view for coverage to the sidewalls and the hairline gap, side view for a thin even coat with no pooling at the cuticle, and the free edge for the cap. Done in that order on every coat.

Original illustration © The Looking Glass Beauty School, 2026.

> ***PRODUCT ON SKIN IS NEVER CURED — If any product has touched the fold or the sidewall skin, remove it with a clean orange stick or a brush dampened with cleanser before the cure. Cured product on skin lifts, looks untidy and sensitises; it is the single most common fault the assessor will look for.***

Nail shapes

Gel polish follows the shape you file into the natural nail. Because it adds no length, the shapes available are the ones the client's own nails have the length for. File the shape at the preparation stage, before any product, and make it match on all ten nails: the eye reads an uneven set of shapes long before it reads an uneven colour.

Shape	Note for a gel polish service
Square	Corners are vulnerable on a natural nail; keep the length short enough to survive the client's work and cap the corners carefully.
Squoval	Softened corners reduce snagging while keeping the look of a square. A practical default.

Round and oval	Naturally strong shapes that follow the finger; the easiest to cap cleanly.
Natural short	The strongest choice and the one that grows out best. Ideal for a client with short or bitten nails.
Almond	Needs natural length to taper into; do not thin the sidewalls to create it.
Coffin, ballerina and stiletto	Enhancement shapes. On a natural nail they are only possible with length the client already has, and they break first.

Figure 16. Nail shapes on the natural nail (top view)

Gel polish follows the shape you file into the natural nail. The free edge is the pale part standing past the fingertip.

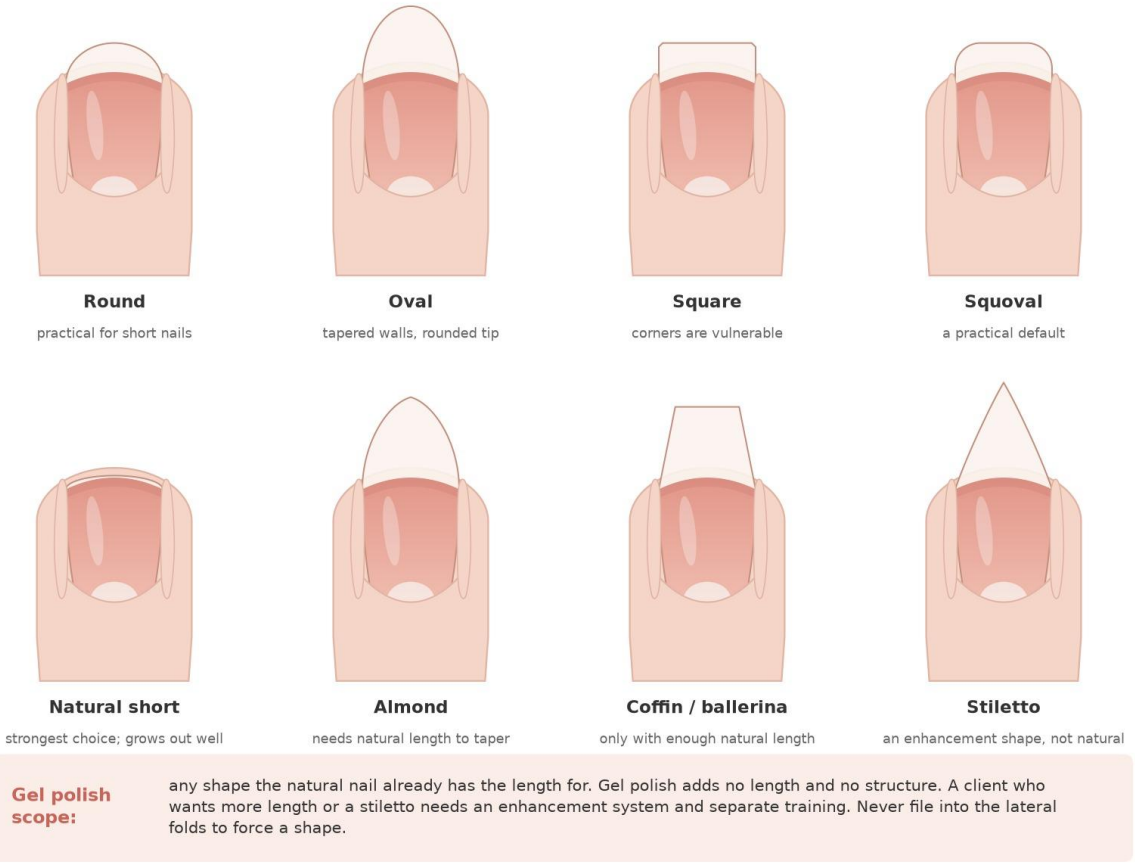


Figure 16. Nail shapes on the natural nail from above. Any shape the natural nail already has the length for; gel polish adds no length and no structure, so length belongs to an enhancement system and separate training.

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Chapter 8 — Kit, equipment, files and product storage

Select the right tool, use it lightly and maintain it correctly

Item	**Purpose and safe use**

Base coat, colour and top coat	One compatible system. Record the exact product names and batch numbers. Keep bottles closed and away from daylight and the lamp — ambient light begins to cure product in the bottle, and a bottle left open beside a lamp thickens within a session.
Compatible lamp	Must match the system's photoinitiators. Record make, model, wattage and the cure schedule for each coat, including any longer time for dark or heavily pigmented colours. A low-heat setting is useful for a client who feels warmth.
Polish-style brushes	The brushes in the bottles. Wipe the neck of the bottle before closing; never cross-contaminate base, colour and top coat by using one brush in another bottle.
Flat clean-up brush and cleanser	For lifting product off skin before the cure and for cleaning a French smile line. Use only the cleanser the system specifies.
Nail prep, dehydrator and (if the system uses one) bonder	Applied to the plate only, immediately before base coat.
Files and buffers	180 grit for the free edge; 240 grit or finer for de-shining; nothing coarser than 240 touches the natural nail. Single-client use or fully decontaminated.
Cuticle tools	Rubber-tipped or wooden pusher for the fold; nippers only for loose, non-living tissue that cannot be lifted away.
Lint-free wipes	Single use, one nail at a time, for the inhibition layer, cleansing and removal.
Removal kit	Acetone, lint-free pads, foil or removal clips, an orange stick or metal pusher used flat, cuticle oil.
Local extraction	Where the risk assessment identifies dust or vapour risk; the filing in a gel polish service is light, but removal and preparation still create dust.

Files and grits

Grit	Category	Typical use in a gel polish service
100/180 combination	Coarse / medium	Old enhancements only (acrylic, hard gel). Not part of a gel polish service; never on the natural nail.
180	Medium	Shaping and shortening the free edge, finger supported; breaking the seal of a cured top coat before a soak-off.
240 and finer buffers	Fine to very fine	Removing surface shine from the natural nail before base coat, with light strokes; lifting the last flakes after a soak-off.
Shine buffer	Very fine	Not needed under gel polish, which takes its shine from the top coat. Used after removal when a client leaves bare.



Figure 17. Files and buffers for a gel polish service. The natural nail meets nothing coarser than 240 grit, and then only to remove shine.

Original illustration © The Looking Glass Beauty School, 2026.

LIGHT AND HEAT SPOIL GEL — Keep bottles closed and out of direct light. Product left open under a desk lamp begins to cure in the bottle and will never perform correctly.

Chapter 9 — Health, safety, hygiene and ergonomics

Preventing exposure, cross-contamination and avoidable injury

Risk assessment and hierarchy of controls

The safest approach removes or reduces a hazard before relying on personal protective equipment. A salon COSHH/risk assessment should consider the product, task, route of exposure, frequency, quantity, ventilation, people at risk and emergency action.

Control level	Gel polish example
Eliminate	Do not stock a recalled/non-compliant product; do not perform an unsafe service.
Substitute	Select a professional compliant system with clear safety information where this reduces risk, while recognising that 'HEMA-free' is not allergy-free.
Engineering control	Use suitable local exhaust/downdraught extraction for dust and adequate general ventilation.
Administrative control	Training, consultation, thin application, closed containers, cleaning schedules, limited exposure time and incident reporting.

PPE	Task-appropriate gloves, eye protection and apron where the assessment/SDS requires them. PPE is the last line, not the only control.

Routes of exposure

Route	Examples	Control
Skin	Uncured gel, inhibition layer, cleanser, acetone, contaminated gloves	No skin contact; controlled application; prompt washing; suitable gloves; hand care
Eyes	Splash, dust, contaminated fingers	No touching face; eye protection for splash/dust tasks; eyewash access; follow SDS first aid
Inhalation	Fine filing dust, vapours and aerosols	Avoid unnecessary dust; local extraction; ventilation; closed containers; suitable RPE only where risk assessment requires
Ingestion	Eating/drinking at workstation, contaminated hands	No food/drink/smoking in treatment area; wash hands before breaks
Injection/cut	Nipper/file injury or contaminated sharp	Protect skin, use tools correctly, stop bleeding service and follow blood-spill procedure

Cleaning, disinfection, sanitation and sterilisation

These terms are not interchangeable. Cleaning physically removes dirt and organic matter and must occur before effective disinfection or sterilisation. Sanitation reduces contamination to an acceptable routine level. Disinfection inactivates many disease-causing microorganisms but may not destroy bacterial spores. Sterilisation is a validated process intended to destroy all viable microorganisms, including spores.

Item/method	Correct approach
Hands/skin	Wash with liquid soap and water when visibly dirty and at the start/end of work; dry thoroughly. Use an appropriate hand rub on visibly clean hands where policy allows. Moisturise between services/after work to protect the barrier.
Work surface and lamp exterior	Clean visible soil, then apply a compatible disinfectant for the stated contact time. Do not spray liquid into electrical equipment.

Porous files, buffers, orangewood sticks	Treat as single-client/single-use unless the manufacturer and local policy provide a validated decontamination route. Do not share between clients.
Metal pusher, nippers, clippers	After every client: safely transport as used, clean with detergent/mechanical action, rinse/dry, inspect, then disinfect using a compatible product at correct dilution/contact time. Sterilise by validated steam autoclave if the risk assessment/local authority requires it.
Blood-contaminated reusable tool	Stop, wear appropriate PPE, quarantine safely and follow the blood/body-fluid procedure. Dispose as appropriate or use a validated cleaning and steam-sterilisation route; do not simply wipe and reuse.
Autoclave	Steam steriliser used only for compatible instruments with validated cycles, packaging/loading, records, routine tests and maintenance. It is the recognised salon method when sterilisation is necessary.
UV cabinet/light box	Storage aid for already clean/decontaminated instruments. It must not be described as a reliable steriliser.
Glass-bead unit, hot-air oven, boiling water	Do not present as validated salon sterilisation. Current local-authority guidance commonly states that glass-bead units, UV boxes, hot-air ovens and boilers must not be used for sterilisation.
Chemical immersion	Usually disinfection in salon use. A product may make a sterilant claim only under specific validated conditions; follow label, COSHH and local rules and do not casually call a disinfectant 'sterilising solution'.

Standard between-client sequence

1. Remove used disposables and place them directly into the correct waste container.
2. Move reusable tools to a designated closed 'used' container; keep them separate from clean tools.
3. Clean and disinfect the workstation, hand rest, lamp touch points and any reusable containers using the correct contact time.
4. Perform the full tool-cleaning and disinfection/sterilisation process. Dry, inspect and store protected.
5. Wash and dry hands, replace couch roll/towel and set out only the clean items needed for the next client.

6. Record completed scheduled cleaning and any contamination incident.

Blood or skin-break procedure

1. Stop the service and move products/tools away.
2. Ask the client to apply pressure with clean disposable gauze where appropriate; the professional should wear suitable disposable gloves.
3. Do not continue gel application over broken skin or active bleeding.
4. Dispose of contaminated single-use materials in the route identified by the local waste assessment. Quarantine reusable tools for validated processing.
5. Clean and disinfect the contaminated surface using the salon's blood-spill procedure and product instructions.
6. Wash hands after glove removal, document the incident and provide appropriate first-aid/referral advice.

Ventilation, dust and odour

HSE guidance for nail bars and beauticians (HSE COSHH Essentials sheet SR13, "Nail bars and other beauty services") identifies dermatitis, skin allergy, asthma and respiratory symptoms as relevant risks from beauty products and artificial-nail dust. General ventilation helps, but opening a window alone may not control dust at the technician's breathing zone. Use source capture where the assessment requires it, maintain filters and use methods that create the least dust.

- Keep bottles closed when not dispensing and clean spills immediately according to SDS.
- Position local extraction close enough to capture dust before it crosses the breathing zone.
- Do not rely on a strong smell to judge danger; some hazardous exposure has little odour, and smell perception varies.
- Do not use an ordinary desk fan that blows dust and vapour across the client or room.
- Empty/maintain extraction and waste in accordance with instructions while wearing any identified PPE.

Ergonomics and prevention of backache/RSI

Area	Good practice
Chair and feet	Use an adjustable supportive chair. Sit back with feet flat or on a footrest; avoid perching or twisting.
Desk and elbows	Use a narrow, stable manicure desk so client and technician do not reach. Support both forearms; aim for relaxed shoulders and elbows near 90 degrees.
Neck and vision	Bring the client's hand into the lit working area instead of bending the neck toward the desk. Consider task lighting or magnification.
Wrist and grip	Keep wrist as neutral as possible, use a light tool grip, support the finger and vary

	movement. Replace worn files rather than pressing harder.
Client position	Seat the client squarely facing the desk with supported arm and relaxed hand. Ask them not to pull, twist or hold the fingers rigid.
Work pattern	Insert microbreaks, stretch hands/forearms/neck, alternate tasks, schedule realistic service gaps and review symptoms early.
Warning signs	Persistent pain, tingling, numbness, weakness or night symptoms require workload review and appropriate medical/occupational-health advice.

30-SECOND POSTURE RESET — Feet supported; hips back; shoulders down; client close; forearms supported; wrist neutral; lamp and extraction correctly positioned. Reset between every client.

Chapter 10 — UK legislation, insurance, data and advertising

How the rules connect to everyday salon decisions

Legislation changes and local requirements differ. The following summary explains relevance to gel polish work in Great Britain as at October 2026; it is educational guidance, not legal advice. Businesses must check current official sources, insurer terms and their own local authority before delivery and at each document review.

Core legislation and guidance

Law or requirement	**Relevance to a gel polish professional**
Health and Safety at Work etc. Act 1974	Creates general duties to protect employees and others, including clients, so far as reasonably practicable. In practice: risk assessments, safe systems, training, maintained equipment, consultation and incident action.
Control of Substances Hazardous to Health Regulations 2002 (COSHH)	Requires employers to assess and prevent or adequately control exposure to hazardous substances. Keep SDS, identify skin/inhalation/eye/fire risks, use control measures, train staff, manage spills and review assessments when products or processes change.
Personal Protective Equipment at Work	Where risk remains after other controls,

Regulations 1992, as amended in 2022	employers must provide suitable PPE free of charge, train workers and maintain it. The 2022 amendment extends relevant duties to limb (b) workers. PPE selection must follow the hazard and SDS, not salon fashion.
Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013 (RIDDOR)	Specified work-related injuries, dangerous occurrences and diagnosed occupational diseases must be reported by the responsible person. Not every chip, rash or complaint is RIDDOR-reportable. Work-related diagnosed dermatitis can be reportable where the work involves significant/regular exposure to skin irritants or sensitisers. Use HSE's current decision guidance.
Data Protection Act 2018 and UK GDPR	Client contact details, consultation records, health information and photographs are personal data; health data is special-category data. Identify an Article 6 lawful basis and an Article 9 condition, give privacy information, minimise collection, secure records, set justified retention periods, support rights and separate treatment consent from marketing consent.
Cosmetic Products Enforcement Regulations 2013 and retained Cosmetic Regulation 1223/2009, as amended	Cosmetic nail products supplied in GB must meet safety, responsible-person, labelling and notification requirements. Buy through reputable compliant supply chains, keep original labels/batches, act on recalls and do not manufacture, relabel or import products without understanding responsible-person obligations.
Cosmetic Products Regulation (Restriction of Chemical Substances) (Amendment and Transitional Provisions) Regulations 2026 (SI 2026/23, as supplemented by SI 2026/109) — TPO	GB restrictions affecting Diphenyl(2,4,6-trimethylbenzoyl)phosphine oxide (TPO) begin on 15 August 2026 with transitional provisions for products already placed on the market. Existing compliant stock may continue to be made available through 14 February 2027 under the regulation. TPO has already been banned in the EU and Northern Ireland since 1 September 2025. Because interpretation of salon use and supply can be fact-specific, check current OPSS/Trading Standards and supplier guidance before purchase or use.
Local Government (Miscellaneous Provisions) Act 1982	In England outside London, local authorities can adopt licensing/registration provisions for specified treatments and may impose

	local byelaws or special-treatment conditions. Coverage differs between authorities, so each business and learner must check the authority where they practise and meet insurer requirements.
London Local Authorities Act 1991	In London, premises offering nail treatments generally need a Special Treatment Licence, applied borough by borough with local conditions on qualifications, hygiene and premises. Check the relevant borough before trading.
Civic Government (Scotland) Act 1982	In Scotland, licensing of relevant treatments and premises operates under this Act through the local licensing authority. Check the authority where you practise.
Public Health (Wales) Act 2017	In Wales, this Act established a special-procedures licensing framework. Nail services are not currently within the mandatory licensing categories, but practitioners in Wales must check current Welsh Government and local-authority guidance.
Electricity at Work Regulations 1989	Electrical systems and work equipment must be maintained to prevent danger and used safely. Visually check lamps/leads/plugs, keep liquids away, remove faults from use and arrange competent inspection/testing based on risk. The law does not automatically demand annual PAT testing for every appliance.
Waste Electrical and Electronic Equipment Regulations 2013 (as amended)	Nail lamps and other electrical equipment must enter an appropriate WEEE reuse/recycling route when discarded. Do not place electrical items in normal waste; use supplier take-back or an authorised business-waste route and retain records where required.
Consumer Rights Act 2015	Services must be performed with reasonable care and skill, information relied on by the consumer can become binding, and pricing must be clear. Avoid unrealistic guarantees and maintain complaint/rectification procedures.
Employers' Liability (Compulsory Insurance) Act 1969	Most businesses employing staff must hold employers' liability insurance meeting legal requirements and display/make the certificate accessible. This is separate from public and treatment liability.

Workplace (Health, Safety and Welfare) Regulations 1992	Relevant to safe, clean and suitable workplaces, including ventilation, lighting, temperature, welfare and workstation conditions.
Equality Act 2010	Service providers and educators must not unlawfully discriminate and should consider reasonable adjustments. Adjustments can change access or assessment method but cannot remove essential safety and competence standards.

Whatever the nation, always confirm licensing and registration requirements with your own local authority and your insurer before trading.

Public liability, treatment risk and product liability

Public liability insurance is not one of the Acts listed above, but it is an essential business control and may be required by venues, landlords, local authorities, training providers or contracts. A nail professional should confirm that cover includes the exact treatments, products, teaching activities, students/models, premises and working arrangements. Treatment/professional liability and product liability may be included or separate. Policy conditions often set qualification, age, consultation, record-retention, patch-testing and incident-notification requirements.

- Never assume the salon policy covers a self-employed professional or educator; obtain written confirmation.
- Notify the insurer promptly about an adverse reaction, injury, complaint or potential claim according to the policy. Do not admit liability or alter records.
- Keep factual, contemporaneous treatment and training records for the period agreed with the insurer and documented in the business retention schedule.
- If employing anyone, confirm employers' liability and health-and-safety duties in addition to public/treatment cover.

Data protection in consultation and case studies

Principle	**Salon action**
Transparency	Give a privacy notice explaining controller identity, purposes, lawful basis/condition, recipients, retention, security and client rights.
Data minimisation	Ask only what is necessary for safe treatment. A full GP address or unrelated medical history is normally unnecessary.
Health data	Document the relevant Article 6 lawful basis and Article 9 condition. Explicit consent may be available where genuinely appropriate,

	but treatment consent and data-processing conditions are different legal concepts.
Security	Lock paper files; use access-controlled, backed-up systems; avoid client details in personal messaging/photo galleries; restrict staff access.
Accuracy	Confirm changes at every appointment and record what the client said without turning observations into a diagnosis.
Retention	Use a written schedule based on purpose, insurer/legal advice and limitation periods; review and securely delete/anonymise when no longer needed.
Photographs	Use separate optional consent. State educational/assessment/marketing uses individually, avoid identifying features where possible and never post a case-study image automatically.
Breach response	Contain, assess, document and report qualifying personal-data breaches to the ICO within required timescales where applicable; inform affected people when high risk requires it.

Advertising Standards Authority (ASA) and CAP guidance

Advertising includes websites, booking pages, social media, paid posts, influencer content, leaflets and claims made in before/after images. The ASA/CAP position is that beauty marketers must hold evidence for objective efficacy claims and must not exaggerate effects.

Avoid	**Use instead**
'Guaranteed chip-free for 14 days'	'Designed for longer wear than traditional lacquer; individual wear varies with nail condition, lifestyle and aftercare.'
'Repairs weak or damaged nails'	'Protects the natural nail while it grows; it does not repair, strengthen or treat the nail itself.'
'Completely safe', 'non-toxic' or 'allergy-free'	Describe specific product information accurately and acknowledge that adverse reactions are possible.
'Treats fungus', 'kills infection' or 'cures green nail'	State that suspected infection cannot be treated cosmetically and requires medical/pharmacy advice.
'Qualified nail technician' based only on this CPD activity	'Trained in gel polish application (CPD)', with the underlying qualification described accurately.
Heavily retouched before/after images or	Use comparable lighting/angle, retain

inconsistent lighting	originals, obtain consent and disclose relevant editing/filters.
'Best', 'number one' or award claims without evidence	State the exact verifiable award, year and category; do not imply a wider win than was achieved.

ASA RESOURCE — Learners should review the current ASA/CAP 'Beauty and Cosmetics: General' guidance before advertising: https://www.asa.org.uk/advice-online/beauty-and-cosmetics-general.html

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Electrical and lamp checklist

- ☐ The lamp is obtained from a reputable supplier and is approved for the gel system.
- ☐ Plug, cable, adaptor, housing and timer show no damage, heat marks or exposed parts.
- ☐ The interior is clean and reflective surfaces/sensors are unobstructed.
- ☐ The equipment is kept away from liquids and connected without unsafe adaptors or trailing-trip hazards.
- ☐ User checks, inspection/test intervals and maintenance are based on risk and recorded; an arbitrary annual sticker is not treated as the only control.
- ☐ Faulty equipment is unplugged, labelled and removed from use pending competent repair or WEEE disposal.

Chapter 11 — Preparing the natural nail

Adhesion is won or lost here

1. Sanitise your hands and the client's hands, and inspect every nail and the surrounding skin.
2. Remove any existing product by the method it needs (Chapter 14). Do not apply gel polish over old product of unknown origin.
3. Shorten and shape the free edge to the agreed length with a 180 grit file, supporting the finger.
4. Gently push back the proximal nail fold with a rubber-tipped or wooden tool. Do not cut living tissue.
5. Remove only loose, non-living cuticle tissue adhering to the plate.
6. Remove the surface shine with a fine buffer, 240 grit or finer, using light strokes. This is de-shining, not thinning.
7. Remove dust with a clean brush, then cleanse the plate with the specified product.
8. Apply dehydrator to the plate only and allow it to flash off.
9. If the system uses a bonder or primer, apply it sparingly, to the plate only, exactly as the manufacturer instructs. Many gel polish systems need none; do not add one from another brand.

10. Work one hand at a time so prepared nails do not re-oil before product is applied.

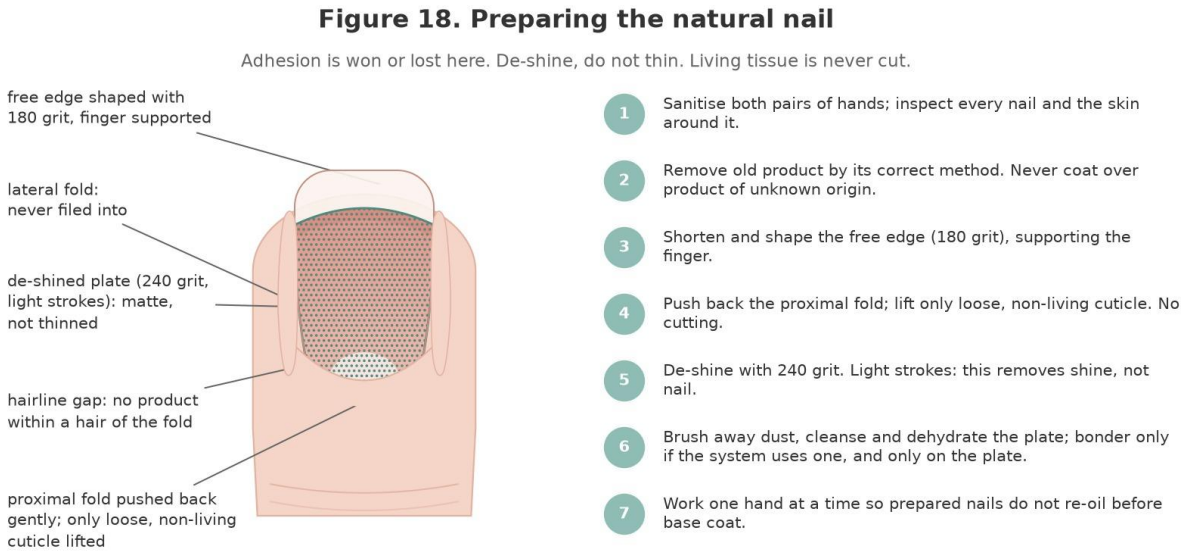


Figure 18. Preparing the natural nail: the free edge shaped, the proximal fold pushed back, only loose non-living cuticle lifted, the plate de-shined with a fine buffer, and a hairline gap kept at every fold.

Original illustration © The Looking Glass Beauty School, 2026.

Preparation fault	Consequence
Cuticle tissue left on the plate	Lifting at the base within days.
Oil or hand cream on the plate	Immediate adhesion failure.
Over-buffed, thinned plate	Sensitivity, weakness, and a higher chance of the client feeling heat during the cure.
Product on living skin	Irritation and a direct route to sensitisation.

Chapter 12 — Applying gel polish: base, colour and top coat

Thin, capped, clean and cured: the same four words for every coat

The three-stroke method

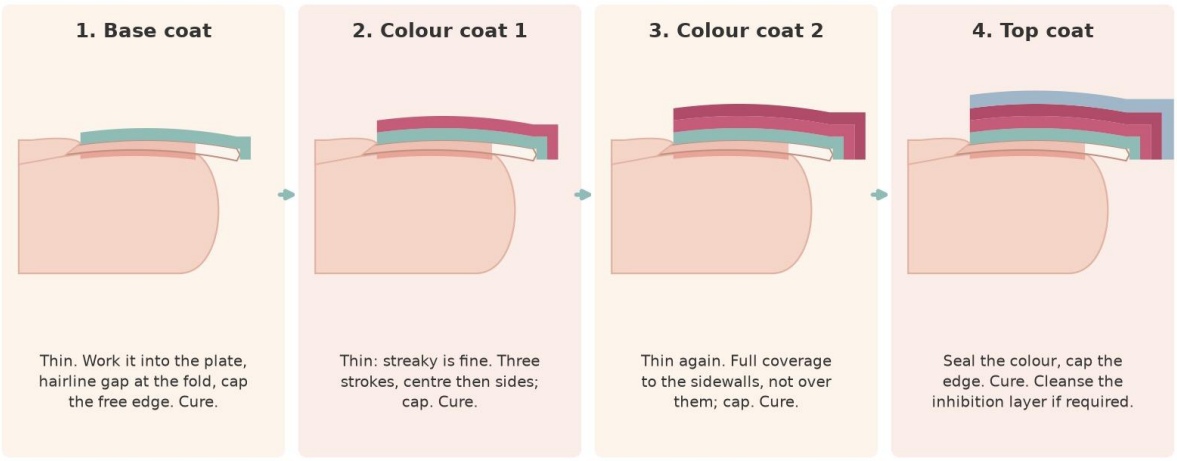
Every coat is applied the same way. Load the brush lightly and wipe one side on the neck of the bottle. Place the brush a hair short of the proximal fold in the centre of the nail and draw it to the free edge in one stroke. Place it again at the fold on one side, push gently towards the sidewall so the product fans into the corner, and draw down; repeat on the other side. Finish by running the last of the brush across the free edge to cap it. Three strokes and a cap. If the coat looks streaky, that is correct; the second coat of colour gives the depth, not a thicker first one.

The sequence

1. Prepare both hands as Chapter 11 describes and check every plate is clean, dry and de-shined. Work one hand at a time from here.
2. Base coat: a thin coat worked into the plate with the three-stroke method, a hairline gap at the fold, the free edge capped. Check for skin contact; cure for the manufacturer's time.
3. Colour coat 1: thin. Streaky is acceptable; thick is not. Cap the free edge. Check the margins; cure.
4. Colour coat 2: thin again, taken to the sidewalls but never over them. Cap. Check; cure. Dark and heavily pigmented colours may need the longer cure time the manufacturer gives for them.
5. Top coat: a thin, even coat over the whole nail, capped, with no bare patch at the edges. Check; cure for the full time.
6. If the top coat leaves an inhibition layer, cleanse it with a fresh lint-free wipe and the system's cleanser, one wipe per nail. A no-wipe top coat is left alone.
7. Repeat on the other hand, then cure the thumbs separately with the nail facing the light.
8. Inspect every nail from above and from the side in good light. Apply cuticle oil to the skin only, and record the products, batches, lamp, cure times and any adaptation.

Figure 19. The application sequence: base, colour, colour, top

Thin coat, cap the free edge, check for skin contact, cure for the full time. Then the next coat.



Three strokes per coat: one down the centre stopping a hair short of the fold, then one each side pushed up to the sidewall. Cap the free edge with the last of the brush.

If any product touches skin, remove it before the cure. Skin contact with uncured product is how allergy starts.

Figure 19. The application sequence: base coat, colour coat 1, colour coat 2, top coat. Each coat is thin, capped at the free edge, checked for skin contact and cured for the full manufacturer time before the next.

Original illustration © The Looking Glass Beauty School, 2026.

THE HAIRLINE GAP — Product that touches the proximal or lateral folds will lift there within days and is a direct sensitisation risk every time it happens. Leave a deliberate hairline gap, and if product creeps into the

fold, lift it off with the clean-up brush before the cure. A tidy gap is the mark of a professional; a flooded cuticle is the mark of a home kit.

In the lamp

The lamp cures what it can see. A tilted hand, fingers pressed together, a tucked thumb, a ring or a sleeve in the way all leave a nail under-cured, and an under-cured nail looks perfect on the day. Place the hand flat on the base of the lamp with the fingers slightly apart and the nails level under the light; cure the thumb on its own with the nail turned up to the light. Use the full manufacturer time for every coat and never shorten it; if a client cannot hold the hand flat, cure fewer nails at a time rather than curing all of them badly.

Figure 20. In the lamp: hand placement decides the cure

The lamp cures what it can see. A tilted hand, a tucked thumb or a shadow is an under-cured nail.

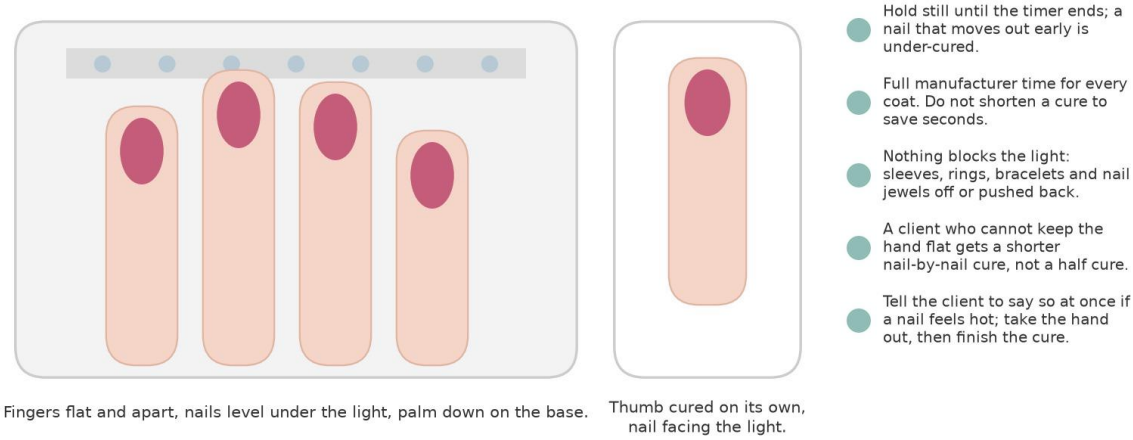


Figure 20. In the lamp: fingers flat and apart with the nails level under the light, the thumb cured on its own with the nail facing the light, nothing blocking the light, and the hand still until the timer ends.

Original illustration © The Looking Glass Beauty School, 2026.

Reading the product as you work

What you see	What it means	What to do
Product floods the cuticle area	Too much on the brush, or the brush placed too close to the fold	Lift it off with the clean-up brush before the cure; load less and start further from the fold on the next nail.
The coat looks thick or domed	Too much product; it will wrinkle and under-cure	Wipe the nail back with cleanser and recoat thinly. Do not cure it and hope.
Streaks or bare patches after the first colour coat	Normal for a thin first coat	Cure it and let the second coat give the coverage. Do not go back over wet product.
Colour pulls away from the	Plate not de-shined there, or	Cleanse and de-shine that area

free edge or sidewall	oil on the plate	again before continuing; check the preparation on the remaining nails.
Surface wrinkles in the lamp	Coat too thick, or the lamp too far from the nail	Remove that coat and recoat thinly; check hand placement.
Top coat dull or tacky after the full cure	Under-cure, an incompatible top coat, or light-degraded product	Re-cure once; if it stays dull, remove the top coat, check lamp and product, and recoat.
Client reports heat	Coat too thick, a thin plate or a powerful lamp	Hand out of the lamp at once; cool; finish the cure; thinner coat next nail. See Chapter 6.

Chapter 13 — Finishes: French, layering and simple nail art

The variations a gel polish professional is expected to offer

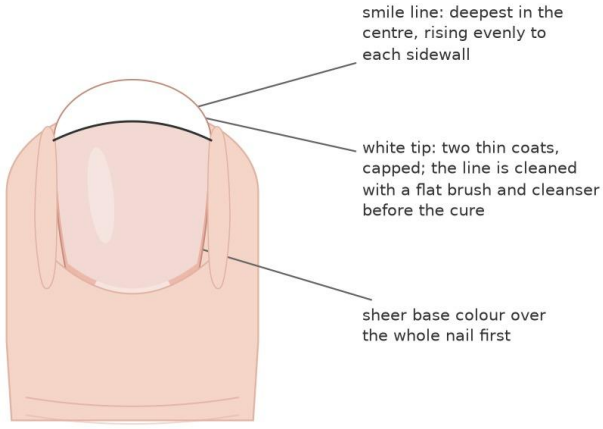
The French finish

A French finish is a sheer or nude colour over the whole nail with a white free edge, divided by a curved smile line that follows the natural one. It is the most requested variation and the one that shows technique most clearly, because the line is drawn freehand on ten nails and the eye compares every one of them.

1. Base coat as Chapter 12, cured.
2. Two thin coats of the sheer colour over the whole nail, each capped and cured.
3. White: load the brush lightly and place it at one sidewall at the depth of the smile line. Draw the curve to the centre in one movement, then from the other sidewall to meet it; fill the tip towards the free edge and cap it. Keep the white thin; two thin coats of white are better than one thick one.
4. Before the cure, tidy the smile line with a flat clean-up brush barely dampened with cleanser, lifting the white back to a clean curve. Check the corners match on both sides; cure.
5. Second coat of white if the coverage needs it, cured, then top coat over the whole nail, capped and cured.

Figure 21. French finish: the smile line

A French tip is two colour coats of sheer plus a white free edge. The smile line follows the natural one.



Three faults to look for

Uneven corners: one side higher than the other.

A flat line: no curve, so the nail looks shorter and wider.

A thick tip: white built up in three coats instead of two thin ones; it chips first.

Figure 21. The French finish: a sheer colour over the whole nail, a white free edge, and a smile line that is deepest in the centre and rises evenly to each sidewall. Uneven corners, a flat line and a thick tip are the three faults to look for.

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Layering and sheer colour

Because each coat is thin and cured, colours can be layered to make new ones: a sheer over a solid shade changes its depth, a white under a colour brightens it and a black under a colour deepens it. Layering is a legitimate way to offer more shades from a small collection, within two rules. Every product in the stack must belong to the same compatible system, and the total application must still be thin: three colour coats and a top coat is the practical maximum. Record the exact combination on the client card so it can be repeated.

Simple nail art within this course

Art that stays within the scope of this course is art done with gel polish and its own top coat: a glitter or shimmer shade over a colour, a dotted accent nail, a single stripe or a contrasting tip, all cured as coats and sealed under top coat. Loose glitter, foils and gems pressed into the inhibition layer and sealed are also within scope provided the top coat seals them completely and the finished nail is still thin and smooth. Chrome powders, encapsulated designs in builder gel, hand-painted art in gel paint, and anything that adds bulk or needs filing belong to a separate nail art course.

> ***STILL THIN — An accent nail with glitter is still a thin, capped, cured coat. If the art makes the nail thick, rough or uncapped, it will chip first and the client will blame the gel polish.***

Chapter 14 — Maintenance, repairs and removal

Gel polish is removed and reapplied, not infilled

Maintenance

Gel polish is maintained by removal and re-application every two to three weeks. It is not infilled: there is no structure to rebalance, and product applied over a grown-out, worn coat lifts at the join. A client who returns at three weeks with the coating intact and a band of regrowth at the base has had a successful service; the regrowth is the natural nail doing what it should.

Repairs

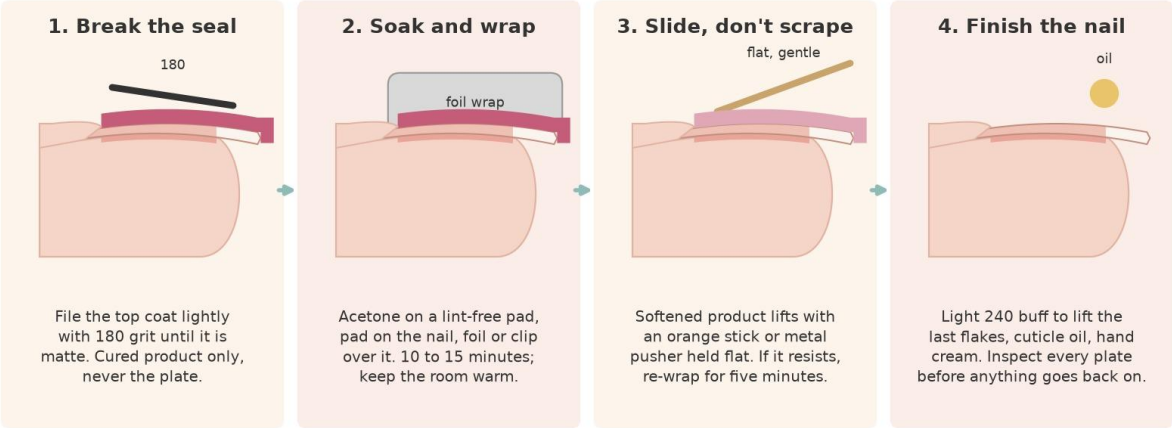
A single nail that chips or lifts in the first few days can be repaired if the cause is clear and the rest of the set is sound. Remove the damaged coating from that nail by soaking (never by peeling the lifted edge), re-prepare the plate, and apply base, colour and top coat as a new nail, matching the colour from the client's record. Look at why it failed before you repair it: a cap missed, a thick coat, oil on the plate, or a client who used the nail as a tool. A set that chips on several nails is not repaired; it is removed, and the preparation and application are reviewed.

Soak-off removal

1. Break the seal: file the top coat lightly with a 180 grit file until the whole surface is matte. Cured product only; stop well before the plate.
2. Protect the skin around the nail with a little cuticle oil or barrier cream if the client's skin is dry.
3. Soak a lint-free pad in acetone, place it on the nail, and wrap the fingertip in foil or fit a removal clip. Work one hand at a time so the other stays usable.
4. Wait 10 to 15 minutes. A warm room and a warm hand speed the process; do not put hands in heated acetone.
5. Unwrap one nail at a time. Softened product lifts away with an orange stick or a metal pusher held flat against the plate. Slide, do not scrape. If it resists, re-wrap that nail for five minutes rather than forcing it.
6. Lift the last flakes with a light 240 grit buffer, then wash the hands, apply cuticle oil and hand cream, and inspect every plate before anything goes back on.

Figure 22. Soak-off removal

Removal is a service, not a chore. Damage clients blame on gel polish is almost always removal damage.



Never peel, pick, prise or file gel polish off the natural nail. Peeling takes the top layers of the plate with it.

Acetone dries skin: oil and cream afterwards. Non-acetone removers do not remove gel polish; do not let a client try.

Figure 22. Soak-off removal in four stages: break the seal of the top coat, acetone pad and wrap for 10 to 15 minutes, slide the softened product off with a tool held flat, then buff lightly and oil. Never peel, pick, prise or file gel polish off the natural nail.

Original illustration © The Looking Glass Beauty School, 2026.

NEVER PEEL — Peeling gel polish off takes the top layers of the nail plate with it, and the thin, rough, white-patched nails that follow are blamed on the product. Tell every client at the first appointment that if a nail lifts, they book a removal; a lifted edge is a reason to come in, not an invitation.

Removing other products

Product on the nail	Method	Warning
Gel polish	Soak-off as above	Non-acetone removers do not work; do not let a client try one at home.
Builder gel in a bottle	Product-specific: soak-off, file-off or a combination as the manufacturer states	Do not assume. If you cannot identify the product and its removal category, do not start; refer to the professional who applied it or to one trained in builder gel.
Hard gel	File-off, by a professional trained in it	Acetone does not remove it. Outside the scope of this course.
Liquid-and-powder acrylic	Reduce bulk by file, then soak, by a professional trained in it	Outside the scope of this course.
Lacquer over gel polish	Non-acetone remover for the lacquer first, then soak-off	Acetone will take both; use it only if the gel polish is also being removed.

KNOW WHAT YOU ARE TAKING OFF — Soaking a file-off product wastes twenty minutes and achieves nothing; filing a soak-off product down to the plate removes nail with it. If the client cannot tell you what is on the nail, the honest answer is a referral, not a guess.

Chapter 15 — Longevity and troubleshooting

Reading a service that has gone wrong

Problem	Likely cause	Prevention
Lifting at the cuticle area	Cuticle tissue left on the plate, product flooded onto skin, or base coat too thick there	Thorough preparation; hairline gap; lift product off skin before the cure.
Chipping at the free edge	Free edge not capped, or the edge shaped after the coats went on	Shape first; cap every coat on every nail.
Coating peels off in a sheet	Under-cured: wrong lamp, shortened cure, thick coat, or the hand not flat	Compatible lamp, full time, thin coats, correct hand placement.
Wrinkled or rippled colour	Coat too thick, or the first colour coat not cured before the second	Thin coats; cure every coat; wipe back and recoat a nail that looks thick.
Colour pulls back from the sidewalls	Oil or dust on the plate, or the area not de-shined	Cleanse and de-shine to the sidewalls; work one hand at a time.
Dull or tacky top coat	Under-cure, incompatible top coat, light-degraded or old product, or the inhibition layer wiped with a dirty wipe	Check lamp and product age; keep bottles closed and out of light; fresh wipe per nail.
Bubbles in the colour	Brush worked too fast, bottle shaken, or product applied over an uncleansed, uncured surface	Roll the bottle; load lightly; apply with slow strokes.
Client reports heat during cure	Coat too thick, thin plate, powerful lamp	Hand out of the lamp at once; thinner coats; low-heat setting if available.
Green discolouration under the coating	Moisture trapped under a lifted area	Never recoat over lifting; follow the referral procedure in Chapter 4.
Client reports itching, redness or swelling around the nails or on the face	Possible acrylate sensitisation	Stop, remove safely by soaking, refer for medical assessment, do not reapply.

Setting expectations honestly

- Two to three weeks of wear is normal; regrowth at the base is the natural nail growing, not a service failure.
- Gel polish protects the free edge so it is less likely to split; it does not make nails grow faster or stronger.
- A client who uses the nails as tools, works in water without gloves or picks at the coating will get less wear, and that is not the product's fault.
- The natural nail is not damaged by gel polish correctly applied and correctly removed. It is damaged by peeling, by scraping and by filing it thin.
- Never promise a number of days of wear. Explain what affects it and record what you advised.

Chapter 16 — Contra-actions, adverse reactions and emergencies

Recognise early signs, stop exposure and escalate appropriately

Contra-action or adverse reaction?

A contra-action is an unwanted response or service problem during or after treatment, such as heat discomfort, minor accidental skin abrasion, lifting or dryness. An adverse reaction is a harmful response to a product or process and may include irritant dermatitis, allergic contact dermatitis, respiratory symptoms, chemical eye exposure or burns. The professional does not diagnose the mechanism; they recognise signs, stop exposure, give proportionate first aid and refer.

Event	Possible signs	Immediate management
Brief warmth/heat spike	Warmth or sharp burning during cure	Ask client to remove hand briefly if burning; stop/reassess; use manufacturer heat protocol; never tell client to tolerate pain
Product on skin before cure	Visible uncured gel in fold/on skin	Remove completely with clean tool/wipe; wash contamination from skin as appropriate; do not cure until skin is clean
Minor skin break/bleeding	Nipper/file injury	Stop; first aid and blood procedure; do not apply gel; document; refer if needed
Irritant contact reaction	Burning, redness, dryness or cracking after chemical contact	Stop exposure; remove wet product; wash with soap/water; follow SDS; advise medical/pharmacy help if persistent

Suspected allergic contact dermatitis	Itching, swelling, redness, blistering, fingertip changes; may affect eyelids/face/neck	Stop all relevant nail product exposure; do not re-test; advise GP/dermatology assessment; document product/lamp/batch
Eye exposure	Stinging, watering, redness or product splash	Follow SDS immediately; irrigate with clean lukewarm water for specified time, remove contact lenses if easy, obtain urgent medical advice
Respiratory symptoms	Cough, wheeze, chest tightness, dizziness or shortness of breath	Stop work, move to fresh air if safe, follow SDS/emergency plan. Call 999 for severe breathing difficulty or collapse
Severe allergic/emergency signs	Rapid facial/throat swelling, breathing difficulty, collapse, confusion	Call 999 immediately; follow emergency/first-aid training; do not leave the person alone
Post-service lifting/chipping	Coating separates or chips	Assess promptly. Remove lifted area/product; inspect for moisture/discolouration; repair only when healthy
Possible infection after service	Increasing pain, heat, swelling, pus, spreading redness or green discolouration	Do not treat or cover; advise urgent pharmacy/GP/NHS 111 assessment; emergency care if systemically unwell

General adverse-reaction response

1. **STOP** the treatment and prevent further exposure.
2. **REMOVE** uncured/wet product safely without spreading it; do not use additional sensitising solvents on skin unless the SDS directs.
3. **RINSE/WASH** or provide first aid according to the exact product SDS and first-aid training.
4. **ASSESS** urgency. Call 999 for emergency signs; otherwise advise NHS 111, GP, pharmacist or occupational health as appropriate.
5. **RECORD** factual signs, times, affected area, product names/batches, lamp/model/cure, PPE, action, advice and witnesses. Photograph only with consent and without delaying care.
6. **REPORT** internally, to insurer and under RIDDOR/OPSS rules where applicable. Preserve product/records; do not alter notes.
7. **REVIEW** the COSHH assessment, product system, lamp, training and technique before any further service. Do not perform a salon 'patch test' to prove safety.

Allergy prevention for professionals

- Keep uncured product off your own skin and the client's skin every time, not only when a reaction is suspected.
- Use controlled dispensers/brushes, clean bottle necks and avoid wiping gel onto gloves, towels or lamp surfaces.
- Change contaminated gloves immediately and select glove material/change intervals from the SDS/COSHH assessment.
- Do not touch your face, phone, pen or door handles with contaminated gloves.
- Use extraction for dust and prevent airborne/residual gel contamination during filing/removal.
- Report hand eczema, cracking, eyelid/facial rash, cough or wheeze early. Once sensitised, continued exposure can jeopardise a technician's career and future medical/dental choices.

Chapter 17 — Aftercare and professional client support

Clear advice protects the service and the natural nail

Aftercare advice

- Apply cuticle oil to intact skin daily and massage around the coating. Stop if irritation occurs.
- Wear suitable gloves for prolonged wet work, cleaning, gardening and chemical exposure; dry hands thoroughly afterwards.
- Do not pick, peel, bite or prise the coating. Do not use nails as tools to open cans, scrape labels or lift objects.
- Keep the chosen length practical. If an edge catches, contact the nail professional; do not tear it or glue a damaged natural nail without advice.
- Book professional removal/reapplication according to growth, condition and product guidance, commonly around 2–3 weeks.
- Contact the salon promptly for lifting, a crack, trapped moisture, green/black/yellow change, unexpected white/powdery area or significant chip.
- Stop product exposure and seek advice for itching, redness, swelling, blistering, burning, fingertip changes or rash on hands/face/eyelids. Seek emergency help for breathing or throat/facial swelling.
- Do not cover a changed nail with ordinary polish or another enhancement before it has been assessed.
- Acetone removal may leave skin/nails temporarily dry; moisturise intact skin and keep nails short if not reapplying.
- Results vary: aftercare supports wear but cannot make gel polish indestructible or guarantee a fixed number of days.

Written aftercare handout

> ***CLIENT TAKEAWAY — LOOK AFTER YOUR GEL POLISH: Oil your cuticle skin daily. Wear gloves for wet and chemical work. Never pick or peel; if a nail lifts or chips, book a repair or removal. Keep nails at a practical length. Book professional removal every two to three weeks; do not use the nails as tools. Contact us for lifting, damage, colour change or discomfort. Stop exposure and seek medical advice for a suspected reaction; call 999 for breathing difficulty, collapse or rapid face/throat swelling.***

Professional follow-up

- Record the advice given and the recommended review/removal date.
- Invite the client to report early problems without blame. Early lifting can be removed before moisture is trapped.
- At the next visit, compare wear with the previous plan and adjust length, shape, preparation or system only for a clear reason.
- Do not repeatedly repair the same failure without investigating product compatibility, equipment, technique, nail condition and lifestyle.
- Document complaints factually, preserve photographs/product records and follow the salon's complaint and insurer-notification procedure.

> ***TEACH-BACK ACTIVITY In pairs, give the client aftercare in 60 seconds. The listener checks that you covered professional removal, wet-work protection, no picking, early reporting of lifting or colour change, and urgent reaction signs. Swap roles and improve one phrase.***

Chapter 18 — Course conclusion and next steps

Bringing the learning together before you work on your own clients

What you have covered

Chapter 1 set out ten learning outcomes. By the end of the taught day, and once your three case studies are complete, you should be able to:

- say what gel polish is for, who it suits, and when builder gel, acrylic or an ordinary manicure is the honest answer (LO1);
- name the structures of the nail unit and recognise the conditions that mean adapt, postpone or refuse (LO2);
- consult, record and gain informed consent, and write a treatment plan another professional could follow (LO3);
- explain how gel polish cures, why it is applied in thin coats, and what lamp compatibility and under-curing mean for the client's skin (LO4);

- select, handle, store and dispose of products and equipment under COSHH and the manufacturer's instructions (LO5);
- prepare the natural nail without damage to the plate or living tissue (LO6);
- apply base, colour and top coat in thin, even, fully cured coats with a capped free edge and nothing on the skin (LO7);
- produce a French finish and simple colour variations to a commercial standard (LO8);
- remove gel polish by soaking without damage, and recognise when another product needs a different method or a referral (LO9);
- give aftercare, manage contra-actions and reactions, and set honest expectations about wear (LO10).

Questions, feedback and reflection

Ask anything you are unsure of before you leave; there is no question too small when the alternative is guessing on a paying client. At the end of the taught day you will complete an evaluation form, and your reflective practice form goes into your portfolio with the case studies; both are in the Tutor and Assessor Pack. Reflection is part of continuing professional development: it is how a day's training becomes a change in the way you work. Your feedback also shapes the next version of this course.

What happens next

- Your CPD certificate is issued by The Looking Glass Beauty School once the theory paper, the practical observation and all three case studies are satisfactory. It carries the course title exactly as accredited and the CPD hours.
- Before offering the service commercially, confirm with your insurer that this CPD course, your underlying qualification and your working arrangements are accepted, and check your local authority's requirements (Chapter 10).
- Complete Form G, the product and lamp compatibility record, for the system you will use in your own salon, before your first paying client.
- Start with plain colour on healthy natural nails. Add French and colour variations once every coat is thin, even and capped without thinking about it.

Continuing your development

Gel polish is the colour service every other nail system builds on. The natural next steps at The Looking Glass Beauty School are the Builder Gel Overlay and Short Extension CPD Course, for clients who want strength and a true apex on the natural nail, and the Liquid and Powder Acrylic Enhancement CPD Course for length and strength beyond what the natural nail offers; nail art follows naturally from the French and layering techniques in Chapter 13. Support continues after the course: contact the school by phone, message or email with questions about products, technique or a client you are unsure about. We would rather hear from you than have you guess. Details of every course and how to book are on the school's website and social media.

THANK YOU — for choosing The Looking Glass Beauty School. Thin coats, full cures, nothing on the skin, and the client's hand out of the lamp the moment it feels warm: the rest is practice.

Chapter 19 — Learner activities and knowledge checks

Work through these before, during and after the taught day

Before the taught day

1. Read Chapters 1 to 10 and note three questions to bring with you.
2. Obtain the Safety Data Sheets for the base coat, colour and top coat you will use. Record the product names, whether they contain HEMA or di-HEMA TMHDC, and the lamp the manufacturer specifies.
3. Find the make, model and wattage of the lamp you will use, and its cure schedule for this product.
4. Write, in your own words, why gel polish is applied in thin coats and what happens to the client's skin when it is not.

Knowledge checks

#	Question
1	Explain what a photoinitiator does and why lamp compatibility matters.
2	Why is gel polish applied in thin coats? Name three things that go wrong with a thick coat.
3	What is the inhibition layer, and when is it removed?
4	What is capping the free edge, and what happens on a nail where it was missed?
5	Why is a hairline gap at the cuticle area correct technique, and what does product on the skin lead to?
6	A client feels heat in the lamp. What do you do, in order, and what do you change for the next nail?
7	Describe soak-off removal step by step and explain why peeling damages the nail.
8	A client has builder gel on from another salon and asks for gel polish. Explain how you proceed.
9	Why is 'Shellac' not used as the course title or on the client's record?
10	A set peels off in a sheet after four days. Give three possible causes and how you would tell them apart.
11	Explain why acrylate sensitisation is a greater risk to you than to your client.
12	A client wants long stiletto nails in gel polish. Explain your answer.

SCENARIO A — Two nails have lifted at the cuticle and one has green discolouration under a lifted corner. Set out your assessment, what you do to each nail, what you say, and what you record.

SCENARIO B — Halfway through a set, a client pulls her hand out of the lamp and says her finger is burning. Describe exactly what you do next, what you change for the remaining nails, and what goes in the record.

SCENARIO C — A client arrives with a chipped set she has been picking at for a week and asks you to 'just go over it'. Explain what you do, what you say, and why.

After the taught day

- Complete three case studies as described in Chapter 1.
- For each, record the product and lamp used, cure times, the structure achieved, the aftercare given and one thing you would do differently.
- Photograph each set from above and from the side, with consent, in the same light before and after.

Chapter 20 — Learner forms and client records

Templates to evidence consultation, planning, aftercare and case studies

DATA PROTECTION These forms collect personal and sometimes special-category health data. Do not ask for information that is not necessary for treatment safety.

Form A — Gel polish consultation and treatment plan

Provider data-protection details

Data controller (school/salon name)	_____
ICO registration reference	_____
Privacy notice location	_____
Lawful basis and special-category condition	_____
Retention period	_____

Template: the school or salon completes this block before the form is used with clients.

Client/model full name	_____
Date of birth / confirmation aged 18+	_____
Telephone or preferred contact	_____
Date and time	_____

Technician/learner	_____

Desired result and lifestyle

Desired colour, finish and shape	_____
Work and hand-use considerations	_____
Wet work/chemicals/sport/hobbies	_____
Previous gel service and typical wear	_____
Previous removal method and any damage	_____

Relevant health, allergy and nail questions

Relevant question	**Yes/No**	**Necessary details**
Have you ever had itching, redness, swelling, blistering, nail lifting or rash after gel, acrylic, nail glue, adhesive or dental material?		
Do you currently have pain, heat, swelling, broken skin, rash, discharge or colour change around any nail?		
Have you been diagnosed with a nail/skin condition that affects this treatment area? If yes, what treatment limitation has been advised?		
Are you taking medication or receiving treatment that has recently changed your skin, nails, healing or sensitivity? Record only relevant details.		
Do you have diabetes, impaired circulation, reduced sensation, immune suppression or another factor that requires extra care?		
Is there anything else relevant to your ability to use the lamp, remain positioned		

or follow aftercare?		

Professional nail and skin assessment

Area	Observation	Proceed/adapt/postpone/refer
Nail plates		
Nail folds/cuticle		
Free edges/hyponychium		
Colour/attachment		
Skin integrity		
Other		

Agreed treatment plan

System and named products	_____
Lamp make/model and cure schedule	_____
Preparation/adaptations	_____
Colour, finish and shape agreed	_____
Removal/review date	_____
Price	_____
Aftercare supplied	_____

Treatment consent: I confirm that I have answered the relevant consultation questions honestly, had the planned gel polish service, foreseeable risks, alternatives, aftercare and costs explained, and had an opportunity to ask questions. I understand that results and wear vary and that I may ask the professional to stop at any time.

Client/model signature and date	_____
Technician/learner signature and date	_____
Tutor signature if assessed	_____

Data notice/condition: I have received or can access the provider's privacy notice. Where the provider relies on explicit consent to process relevant health data, that consent must be recorded separately and must meet ICO requirements.

Relevant data-processing record / explicit-consent statement if used

_____

Form B — Separate photograph consent

Photograph consent is optional and separate from treatment consent. Tick each use agreed. Refusing marketing use must not affect access to treatment.

- ☐ Assessment evidence viewed by The Looking Glass Beauty School educator/quality assurer.
- ☐ Accreditation evidence viewed by the accrediting body where required.
- ☐ Anonymous learner portfolio (no face/name/identifying jewellery).
- ☐ The Looking Glass Beauty School website/social media/marketing.
- ☐ The salon's website/social media/marketing.

How long images will be retained	_____
How consent can be withdrawn for future use	_____
Model name	_____
Model signature/date	_____
Learner/educator signature/date	_____

Form C — Treatment record

Record field	**Details**
Date / technician	
Consultation changes	
Nail condition before service	
Products used: base, colour, top coat (names and batches)	
Lamp model and exact cure times	
Preparation and service steps	
Variations/contra-actions	
Outcome	
Aftercare and review date	
Client comments	
Signatures	_____

Form D — Three-case-study log

Case study 1

Model code/name	_____
Date	_____

Service and goal	_____
Nail/skin assessment	_____
Product system and lamp	_____
Cure schedule	_____
Preparation/adaptations	_____
Outcome and photograph filenames	_____
Aftercare	_____
What went well	_____
What I would improve	_____
Follow-up/wear result	_____
Educator feedback	_____
Result: Satisfactory / Further evidence required	_____

Case study 2

Model code/name	_____
Date	_____
Service and goal	_____
Nail/skin assessment	_____
Product system and lamp	_____
Cure schedule	_____
Preparation/adaptations	_____
Outcome and photograph filenames	_____
Aftercare	_____
What went well	_____
What I would improve	_____
Follow-up/wear result	_____
Educator feedback	_____
Result: Satisfactory / Further evidence required	_____

Case study 3

Model code/name	_____
Date	_____
Service and goal	_____
Nail/skin assessment	_____
Product system and lamp	_____
Cure schedule	_____
Preparation/adaptations	_____
Outcome and photograph	_____

filenames	
Aftercare	_____
What went well	_____
What I would improve	_____
Follow-up/wear result	_____
Educator feedback	_____
Result: Satisfactory / Further evidence required	_____

Form E — Adverse reaction / incident record

Client/learner/staff name	_____
Date/time/location	_____
Treatment stage	_____
Factual signs and symptoms	_____
Affected nail/skin/area	_____
Product names, batches and SDS	_____
Lamp model and cure details	_____
PPE and ventilation in use	_____
Immediate action/first aid	_____
Emergency service/NHS 111/GP/pharmacist advice	_____
Witnesses	_____
Photograph consent/file reference	_____
Insurer/internal/RIDDOR/OPSS notification decision	_____
Follow-up and risk-assessment review	_____
Completed by/signature/date	_____

Form F — Cleaning and equipment log

Date/time	Item/area	Clean/disinfect/sterilise	Product/cycle/contact time	Completed by	Fault/action

Form G — Product and lamp compatibility record

This record is completed twice: first during training, using the system the tutor demonstrates, and again by the learner for their own kit before offering the service commercially.

	Lamp make/model	Output stated	Cure time	Manufacturer evidence/source	Date checked

Form H — Learner declaration

I understand the scope of this CPD activity and will not describe it as a regulated qualification. I will work only within my underlying qualifications, competence, insurer and local requirements; use compatible products and lamps; avoid skin exposure; refuse suspected infection/reaction; and maintain accurate client and product records.

Learner name	_____
Signature	_____
Date	_____
Educator name/signature	_____

Chapter 21 — References and further reading

Authoritative sources used for technical and regulatory checking

Legislation and guidance can change; the provider reviews this list at least annually and whenever a product, lamp, insurer rule, local-authority condition or official safety alert changes.

1. UK Parliament — *Health and Safety at Work etc. Act 1974* — 1974 — https://www.legislation.gov.uk/ukpga/1974/37
2. UK Parliament — *Control of Substances Hazardous to Health Regulations 2002* — 2002 — https://www.legislation.gov.uk/uksi/2002/2677
3. Health and Safety Executive — *COSHH Essentials sheet SR13: Nail bars and other beauty services* — 2025 — https://www.hse.gov.uk/pubns/guidance/sr13.pdf
4. UK Parliament — *Personal Protective Equipment at Work Regulations 1992* (as amended by the Personal Protective Equipment at Work (Amendment) Regulations 2022) — 1992/2022 — https://www.legislation.gov.uk/uksi/1992/2966
5. Health and Safety Executive — *Personal protective equipment (PPE) at work guidance* — 2022 — https://www.hse.gov.uk/ppe/
6. UK Parliament — *Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013 (RIDDOR)* — 2013 — https://www.legislation.gov.uk/uksi/2013/1471
7. Health and Safety Executive — *RIDDOR: occupational diseases* — n.d. — https://www.hse.gov.uk/riddor/
8. UK Parliament — *Electricity at Work Regulations 1989* — 1989 — https://www.legislation.gov.uk/uksi/1989/635

9. Health and Safety Executive — *Portable appliance testing (PAT) FAQs* — n.d. — https://www.hse.gov.uk/electricity/faq-portable-appliance-testing.htm
10. UK Parliament — *Waste Electrical and Electronic Equipment Regulations 2013* — 2013 — https://www.legislation.gov.uk/uksi/2013/3113
11. UK Parliament — *Data Protection Act 2018* (with UK GDPR) — 2018 — https://www.legislation.gov.uk/ukpga/2018/12
12. Information Commissioner's Office — *Special category data: guidance* — n.d. — https://ico.org.uk/for-organisations/uk-gdpr-guidance-and-resources/lawful-basis/special-category-data/
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15. UK Parliament — *Consumer Rights Act 2015* — 2015 — https://www.legislation.gov.uk/ukpga/2015/15
16. UK Parliament — *Cosmetic Products Enforcement Regulations 2013* — 2013 — https://www.legislation.gov.uk/uksi/2013/1478
17. UK Parliament — *London Local Authorities Act 1991* (special treatment licences) — 1991 — https://www.legislation.gov.uk/ukla/1991/13
18. UK Parliament — *Local Government (Miscellaneous Provisions) Act 1982* — 1982 — https://www.legislation.gov.uk/ukpga/1982/30
19. UK Parliament — *Civic Government (Scotland) Act 1982* — 1982 — https://www.legislation.gov.uk/ukpga/1982/45
20. Welsh Parliament — *Public Health (Wales) Act 2017* (special procedures) — 2017 — https://www.legislation.gov.uk/anaw/2017/2
21. European Commission — *Commission Regulation (EU) 2020/1682* (HEMA and di-HEMA TMHDC restriction, professional use only; as retained in the GB Cosmetics Regulation) — 2020 — https://www.legislation.gov.uk/eur/2020/1682
22. European Parliament and Council — *Regulation (EC) No 1223/2009 on cosmetic products* (applies in Northern Ireland) — 2009 — https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX%3A02009R1223-20250901
23. UK Government — The Cosmetic Products Regulation (EC) No 1223/2009 (Restriction of Chemical Substances) (Amendment and Transitional Provisions) Regulations 2026, SI 2026/23 — TPO inserted as entry 1745 of Annex II; regulation 2(3) in force 15 August 2026, with transitional provision to 14 February 2027 — https://www.legislation.gov.uk/uksi/2026/23/made
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29. NHS — *Fungal nail infection* — n.d. — https://www.nhs.uk/conditions/fungal-nail-infection/
30. Advertising Standards Authority / Committee of Advertising Practice — *Beauty and cosmetics: general advice online* — n.d. — https://www.asa.org.uk/advice-online/beauty-and-cosmetics-general.html
31. Cosmetic, Toiletry and Perfumery Association — *Guidance on nail products* — n.d. — https://www.ctpa.org.uk/
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All references checked 26 October 2026 as part of the scheduled document review.